

**Debra A. Pinals, M.D.**

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Board Certified in Psychiatry, Forensic Psychiatry, and Addiction Medicine

**Neutral Expert Eleventh (11th) Report  
Regarding the Consolidated *Mink and Bowman* Cases**

**Date of Report:** May 5, 2025

**Neutral Expert:** Debra A. Pinals, M.D.

**Background and Context of this Report:**

On 12/21/21, The Honorable Michael W. Mosman, U.S. District Judge for the United States District Court for the District of Oregon, Portland Division, entered an order appointing me, Dr. Debra Pinals, as the Neutral Expert in the *Mink/Bowman* matter. The Court's order consolidated two cases, *Bowman et al v. Matteucci et al* (Case Number: 3:21-cv-01637-MO) and *Oregon Advocacy Center et al v. Mink et al* (Case Number: 3:02-cv-00339-MO) and identifies *Mink* as the lead case (see below for summary). Through this consolidation, the *Bowman* case was reassigned from The Honorable Marco A. Hernandez to Judge Mosman. On 3/6/24, the *Mink/Bowman* case was re-assigned to the Honorable Adrienne Nelson.

In total, I have produced and provided ten prior reports to the Court regarding this case, most recently the Tenth Report on 11/12/2024. Those prior reports provide greater detail into the summary of this matter and will not be repeated here, except to outline the basic facts behind this case.

***Background and Summary of the Two Consolidated Cases:***

State defendants were previously found by the Ninth Circuit (*OAC v. Mink, 2003*) to be out of compliance with Constitutional requirements and were ordered to admit individuals found unable to Aid and Assist in their criminal cases to Oregon State Hospital for restoration within seven (7) days of receipt of an order for their commitment to OSH for restoration. In December 2021, after further litigation, the parties entered an interim settlement agreement that involved the appointment of a Neutral Expert to provide recommendations to help achieve compliance with the Ninth Circuit's seven (7) day admission requirement as outlined above.

In a separate litigation, in November 2021, plaintiffs Jarod Bowman and Joshawn Douglas-Simpson brought action against the OSH and Oregon Health Authority (OHA) (plaintiffs were later joined by Metropolitan Public Defender) for failure to timely admit these individuals adjudicated Guilty Except for Insanity (GEI) by the Multnomah County Circuit Court, after The Honorable Nan Waller ordered them to OSH for treatment, without unreasonable delay. After further litigation, The Honorable Marco A. Hernandez, United States District Court Judge, agreed with the defendants that a consolidation of the *Mink* and *Bowman* cases made sense.

**Qualifications to Perform this Work:**

I have worked for over twenty-five years as a clinical and academic and forensic psychiatrist and have functioned for over twenty years in state and local level behavioral health administrative leadership, management, policy and legislative development, clinical treatment, forensic evaluation, and consultative roles across several U.S. jurisdictions. Other details are provided in my First Report.

**Sources:**

Background court and legal documents for this case have been reviewed in prior reports and will not be repeated here. The list below includes legal documents I have reviewed between my last report and this report:

1. Berman v. Oregon State Hospital and OHA Class Action Complaint for Declaratory and Injunctive Relief for Violation of Title II of the Americans with Disabilities Act 42 U.S.C. Section 12131 Et Seq and Violation of 42 U.S.C. Sec 1983 14<sup>th</sup> Amendment, Case No. 6:24-cv-01968-MTK;
2. Motion for a Rule to Show Cause Why Defendants Should Not Be Held in Contempt, and For A Remedial Order, filed by Plaintiffs DRO, and associated filings for January 2025 Status Conference, including expert report by Ira K. Packer, Ph.D.;
3. State's Status Report prepared for the 1/24/25 status hearing;
4. Amici Judges' January 2025 Status Conference Memo, submitted 1/22/25;
5. MPD Partial Joinder to DRO's Motion for Contempt and Remedial Order dated 1/23/25;
6. Status Conference Memorandum of Amici District Attorneys, Crime Victims, and Health Systems, dated 1/23/25;
7. State Defendants' Response to Plaintiff's Motion for Contempt, dated 1/31/25;
8. Plaintiff DRO's Reply to Its Motion For A Rule to Show Cause, dated 2/14/25;
9. MPD's Reply in Support of Partial Joinder to DRO's Motion for Contempt, dated 2/14/25;
10. Declaration of Thomas Stenson, Filed by DRO in Reply to Motion, filed 2/14/25;
11. Transcript of the 1/24/25 status hearing before Judge Nelson;
12. Amici Hospitals' Request for Limited Participation in Contempt Hearing, dated 2/28/25;
13. Amici Judges' March 2025 Evidentiary Hearing Memorandum, dated 3/10/25;
14. Amicus Memorandum for Contempt and Remedial Order Hearing Filed by Washington County, dated 3/11/25;
15. Amicus Memorandum Pre-Hearing Filed by Marion County, dated 3/7/25;

16. Declaration of Chance Wooley in Support of Amicus Washington County Memorandum, dated 3/11/25;
17. Declaration of Matthew Green, for the Defendants, received 3/11/25;
18. Defendants' Exhibit 1: Recommendations Tracker dated 3/12/25 (as well as several iterations of prior drafts given that this has been a working document between the Neutral Expert and the defendants);
19. Opinion and Order for Case No.:3:02-cv-00339-AN (lead), and 3:21-cv-01637-AN (consolidated), signed by The Honorable Adrienne Nelson on 3/20/25;
20. Order by Judge Nelson extending the Court's remedial order until 6/29/25, entered on 3/26/25; and
21. Transcript of the March 12 and 13<sup>th</sup> contempt hearing.

General documents I reviewed since my prior report include the following:

1. OSH Forensic Admission and Discharge Dashboard and Restoration Limit Report produced monthly and reporting date reflecting the month prior to report production;
2. OSH Forensic Admissions and Discharge Bi-Weekly Reports;
3. Average Wait Time Prior to Admission Progress Metrics for Benchmark Goals, produced by OSH monthly;
4. OSH-PSRB conditional release dashboard drafts and currently published data as of February 2025;
5. *Mink & Bowman* Monthly Progress Reports from OHA from December 2024 through April 2025;
6. Miscellaneous case information sent under protective order;
7. Miscellaneous media reports;
8. Mink/Bowman Comprehensive Plan updates on progress;
9. Aid and Assist and GEI/PSRB RFI Full List of Recommendations 11.4.24;
10. OHA summaries of Projected Capacity Increases for licensed and non-licensed housing and facilities;

11. Legislative Behavioral Health Workgroup #1 agenda and attachments sent by Cherryl Ramirez on 11/22/24;
12. Important Correction to the Oregon State Bar Criminal Law Handbook sent by James R. Hargreaves;
13. Miscellaneous communications between plaintiffs and defendants;
14. Miscellaneous filings by Amici for mediation and for hearings before Judge Nelson
15. HB 4092 Oregon CMHP Cost study produced by Optumas dated December 2024;
16. Materials Distributed for the A&A Community Subgroup on 1/7/25, including a report by the Meadows Institute and prior meeting minutes and agenda;
17. OJD Forensic Behavioral Health Workgroup Presentation on Aid and Assist and Civil Commitment from 1/15/25 and OJD Aid & Assist and Civil Commitment Data prepared 12/20/24;
18. Materials distributed for the A&A Community Subgroup for 2/11/25;
19. Court decisions for Maryland, Nevada and Washington as examples of cases similar to Mink/Bowman;
20. Continued Commitments data tracked by DRO January 2025;
21. Health share of Oregon response to PCG report sent by Eric Neiman 12/26/24;
22. HB 2470 contemplating jail restoration and expanded authority of the Office of the Public Guardian;
23. Proposals to the legislature offered by Judge Hargreaves received 2/4/25;
24. Draft Community Restoration Annual Report received 2/12/25
25. Draft Community Restoration Manual received 2/14/25;
26. HB 3051 with Amendments
27. Letter from Legacy, Providence, PeaceHealth, and St. Charles Hospitals requesting to withdraw OAR 309-035-0163;
28. Information on Health-Related Social Needs housing funding and ECMU opportunities sent by Mr. Chance Wooley of Washington County CMHP on 2/27/25;
29. HB 3715 and summary documents sent to me by Judge Hargreaves on 3/2/25;
30. HB 2470, directing OHA to expand OSH capacity;

31. Ready to Place Legislative Proposal suggested by the A&A Legislative workgroup, received 3/14/25;
32. Correspondences with the state regarding jail diversion challenges pursuant to my recommendations as articulated by Ms. Kailana Piimauna on 3/17/25;
33. OHA Behavioral Health Organizational Chart as of 3/1/25;
34. OSH Organizational Chart as of 2022;
35. A&A and PSRB Training PowerPoints produced by OHA;
36. Restoration Limit Impact with Proposed Concept Analysis and Projections produced by OHA;
37. A Supply and Demand Approach to problem solving, authored by Judge Hargreaves, received 3/30/25;
38. FES evaluation interview schedule data with estimates for May and June;
39. ODAA Issue Brief;
40. Mink/Bowman brief timeline produced by OHA and sent to Representatives of the Oregon legislature;
41. NWRRC bed census numbers for April 2025 sent to me by Brian Martinek, 4/23/25;
42. OHA and Aid & Assist Restoration Limits slide deck for legislative presentation;
43. A Mixed Methods Study of Competency Restoration in Oregon produced by the Oregon Public Health Division Program Design and Evaluation Services (PDES), dated September 2023;
44. Proposals and comparisons of proposals for legislative consideration, including OJD proposal for a forensic oversight board, sent to me 5/2/25;
45. CFAA draft language;
46. CCO Care Coordination Policy and Procedure Findings Summary 04232025;
47. Medicaid FFS and Care Coordination report as requested;
48. Preliminary community navigator report draft;
49. Letter to Representative Rob Nosse and other legislators dated 3/27/25 regarding delays in CCO contract updates sent by Director Sejal Hathi, MD MBA;

50. Letter to Representative Rob Nosse regarding HB 2205 A, dated 4/15/25, sent by Interim OHA Director Kris Kautz;
51. Bed capacity screenshots sent by Tom Stenson of DRO on 5/5/25; and
52. Clinical records pertaining to specific deaths and cases that have been brought to my attention in this matter.

In the time since my prior report, meetings and discussions have included the following:

1. Periodic communications with The Honorable Adrienne Nelson throughout this interim period;
2. Dialogue with Magistrate Judge Stacie Beckerman during the time of mediation in early January 2025;
3. Meetings with various OHA and OSH staff, including leadership from the hospital and FES;
4. Regular meetings and several ad hoc meetings and discussions with representatives of Governor Kotek, as well as OHA, OSH, DRO and MPD representatives and leaders both separately and together as well as email communications. In addition, I met with staff from these agencies at various points in this interval period.
  - a. From the Governor's Office:
    - i. Amy Baker, Behavioral Health Initiative Director
    - ii. Constantin Severe, Deputy General Counsel
    - iii. KC Ledell, Behavioral Health Senior Advisor
  - b. From OHA, OSH, the weekly/bi-weekly leadership meetings have included primarily:
    - i. Current administrative leaders including Kristine Kautz, OHA Deputy Director, Ms. Ebony Sloan Clarke, Director of Behavioral Health, OHA, along with Samantha Byers, Lisa Nichols and Bonnie Cappa from OHA HSD/ISU and former OSH Interim Superintendent and Chief Medical Officer Dr. Sara Walker, as well as Dr. Morgyn Beckman and Dr. Andy

Butros of the Forensic Evaluation Services, with Mr. Dave Baden stepping in as of 4/12/25 with the resignation of Dr. Walker. I have also met with Medicaid leadership multiple times, including Shawna McDermott and Holly Heiberg most commonly, and have spoken with Dr. Sejal Hathi, OHA Director at the mediation, at the contempt hearing, and during occasional meetings last fall.

- c. From Oregon Department of Justice (DOJ):
    - i. Carla Scott, DOJ Special Litigation Unit Counsel
    - ii. Jill Conbere, Assistant Attorney General, DOJ
    - iii. Sheila Potter, Deputy Chief Counsel, Special Litigation Unit
    - iv. Kailana Piimauna, Sr. Assistant Attorney General, General Counsel Division
    - v. Melissa Chureau, General Counsel Division
  - d. From Disability Rights Oregon (DRO):
    - i. Emily Cooper, Legal Director
    - ii. Thomas Stenson, Deputy Legal Director
    - iii. Dave Boyer, Managing Attorney
    - iv. Jake Cornett, Chief Executive Officer
  - e. From MPD as plaintiff party, Jesse Merrithew of Levi Merrithew Horst PC
- 5. Conversations at various points with several Amici representatives and their attorneys;
  - 6. Meetings with Ms. Cherryl Ramirez, Director AOCMHP, and representatives of CMHPs across Oregon;
  - 7. Telephone call with Drs. Alexander Miley and Michelle Guyton, of Northwest Forensic Institute, LLC, on 3/9/25; and
  - 8. Discussions with Brian Martinek, Executive Director, Northwest Regional Reentry Center and his staff.

I retained the services of Kirsten Beronio, JD, a Medicaid expert, to help provide insights into potential strategies within Medicaid. I have met with her on several occasions, and she has participated in several Medicaid meetings and reviewed related documentation.

I participated in a mediation meeting between the parties and the amici in January 2025 and I attended a status hearing virtually on 1/24/25. I also observed and testified in the contempt hearing that took place on 3/12/25 and 3/13/25 in front of Judge Nelson. I participated in a tour of OSH's Salem campus and Northwest Regional Reentry Center (NWRRC) on 3/11/24, accompanied by Judge Nelson. Plaintiff Ms. Emily Cooper from DRO joined for both site visits and Plaintiff Counsel Mr. Jesse Merrithew participated in the tour of OSH. I have watched several legislative sessions relevant to this matter by video recording.

### **Glossary of Acronyms and Terms Used in this and Prior Reports**

A&A or AA: Aid and Assist  
APD: Aging and People with Disabilities  
CCOs: Coordinated Care Organizations  
CCBHCs: Certified Community Behavioral Health Clinics  
CFAA: County Financial Assistance Agreements  
CMHPs: Community Mental Health Programs  
CRR: Conditional Release Ready meaning approved by the hospital risk review and ready for review by the PSRB  
DOJ: Department of Justice Oregon  
DRO: Disability Rights Oregon  
ECMU: Extended Care Management Unit  
FES: Forensic Evaluation Services  
GEI: Guilty Except for Insanity  
HLOC: Hospital Level of Care  
IMPACTS: Improving People's Access to Community-Based Treatment, Supports, and Services  
ISU: Intensive Services Unit  
MOOVRS: Multi-Occupancy OSH Vacancy Resource & System Improvement Team  
Mosman Order: Refers to the July 3, 2023 Second Amended Order unless otherwise specified  
MPD: Metropolitan Public Defender  
NWRRC: Northwest Regional Reentry Center  
OCBH: Oregon Council for Behavioral Health  
OCDLA: Oregon Criminal Defense Lawyers Association

ODDS: Oregon Developmental Disability Services  
ODHS: Oregon Department of Human Services  
OHA: Oregon Health Authority  
OPDS: Oregon Public Defenders Services  
ORPA: Oregon Residential Provider Association  
OSH: Oregon State Hospital  
PDES: Program Design and Evaluation Services  
PSRB: Psychiatric Security Review Board  
SHRP: State Hospital Review Panel  
SRTF: Secure Residential Treatment Facility

**Summary of Activities and Updates During this Reporting Period:**

This reporting period covers the time between mid-November 2024 through the end of April 2025. The activities on this case have been numerous. With the state having fallen out of compliance in third quarter of 2024, several issues arose, the most pressing being the need to return to compliance. With that, there was an attempt at mediation regarding remedies the state might take, followed by the plaintiff DRO filing a motion for contempt. The motion was heard by Judge Nelson in March 2025. Plaintiff for MPD joined in part with that motion seeking remedies to return the state to compliance. DRO is seeking significant fines should the state be found in contempt. Additional remedies offered by both plaintiffs are discussed below. The state has worked to defend the motion for contempt, arguing that they have been doing everything possible to return to compliance and that there are factors that are not fully within their control as well (such as legislative decisions for funding and statutory change) that will impact their ability to return to compliance. The Governor's Office has provided written support and has talented staff who understand the behavioral health need and the importance of helping advance her agenda to help the state achieve compliance. To that end, this has been a busy time for OHA leadership and representatives from the Governor's office providing testimony before the legislature on bills that align with recommendations in my 10<sup>th</sup> report.

As all this has taken place, I have met regularly with the defendants and have had countless meetings and conversations to review each of my prior recommendations and continue to

examine whether the tasks recommended are complete and ascertain any next steps. Although MPD was participating in state check-ins for the first part of this reporting period, it was determined that this was not necessary, as meetings with the state required detailed operational discussions. DRO has not been meeting directly with the state since its motion for contempt was filed.

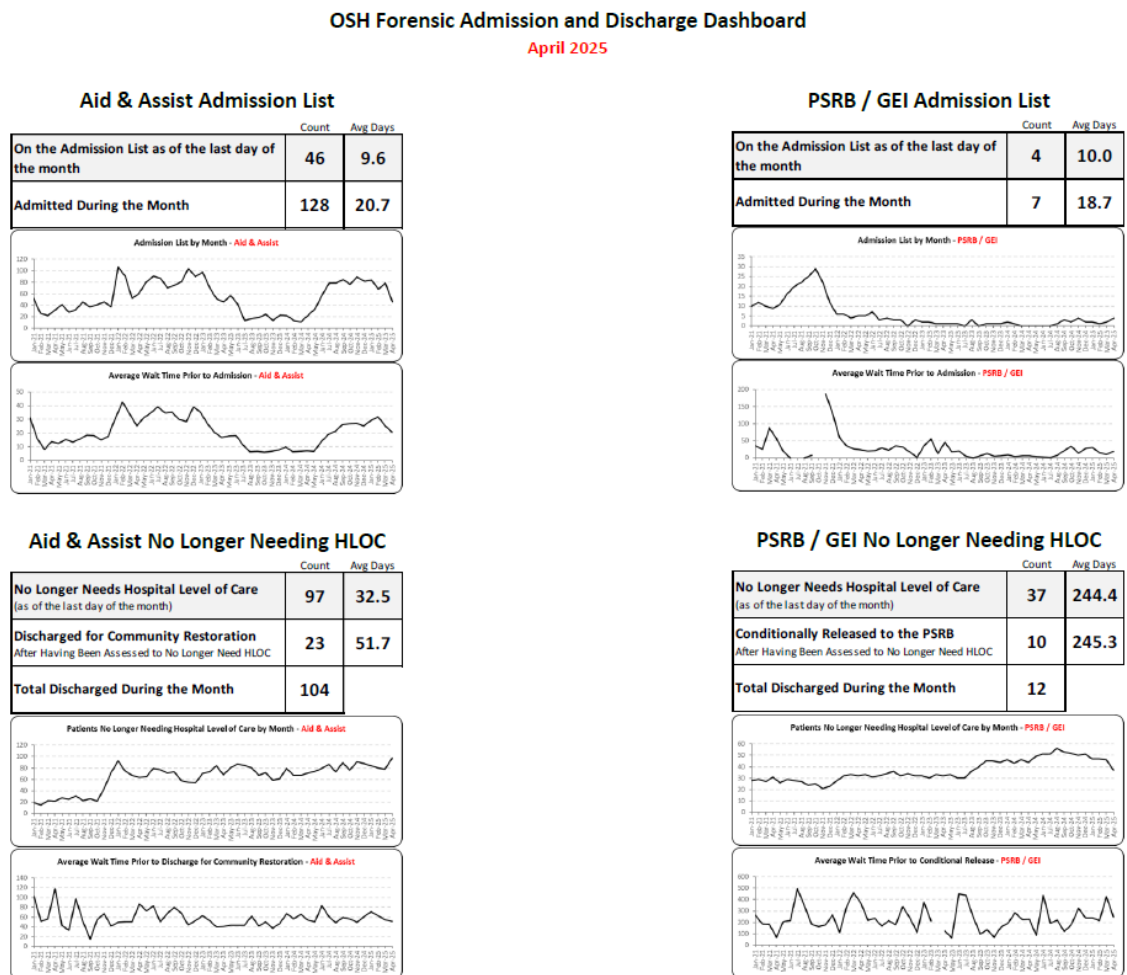
In addition to regular meetings, conversations, data reviews, individual case reviews discussed with the parties, I have watched legislative sessions and continually tracked progress of the state in achieving compliance. This report will summarize information I have gathered and will provide comments and recommendations at this stage.

## **Data**

*Background Data:* The data that is designed to track compliance with the 7-day admissions order is a key metric for this case. Overall, the defendants remain out of compliance with that metric. As of the end of April, 46 people were on the waitlist, with an average active waiting period of 9.6 days (see **Figure 1** and **Table 1**). For those individuals admitted in April 2025, the average days to be admitted was 20.7, down from the prior month of 25.4 days, but still well beyond the 7 days. Still, the numbers are trending down toward compliance. By April 20<sup>th</sup>, it was reported that there were 68 people waiting for admission for an average of 12.1 days, and those that had been admitted between April 7-20<sup>th</sup> had waited 21.8 days. Updated data is depicted in **Table 2**. These numbers are all going in the right direction. On the other hand, the number of people determined as Ready to Place and no longer needing Hospital Level of Care (HLOC) was at 78 at the end of March and by the end of April, was 97 (see **Figure 1**). The GEI admissions waitlist remains fairly constant, most recently with 4 people waiting for admission, and about 37 whom the hospital has determined no longer need hospital level of care.

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**Figure 1. Data Dashboard Charts Reflecting Progress in *Mink/Bowman* as of May 1, 2025**



OSH Quality Management – Data and Analysis  
*‘Informing the Pursuit of Excellence’*

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**Table 1. Individuals Awaiting Admission**

| 1. Regarding individuals on OSH admission list with signed and received A&A court order |                     |                      |                     |                     |                      |                     |                     |                      |                     |                      |                     |
|-----------------------------------------------------------------------------------------|---------------------|----------------------|---------------------|---------------------|----------------------|---------------------|---------------------|----------------------|---------------------|----------------------|---------------------|
|                                                                                         | As of<br>1/5/<br>22 | As of<br>1/28/<br>22 | As of<br>5/1/<br>22 | As of<br>9/1/<br>22 | As of<br>12/1/<br>22 | As of<br>4/1/<br>23 | As of<br>7/1/<br>23 | As of<br>11/1/<br>23 | As of<br>4/1/<br>24 | As of<br>11/1/<br>24 | As of<br>4/1/<br>25 |
| Total Number of individuals                                                             | 46                  | 93*                  | 67                  | 70                  | 104                  | 51                  | 42                  | 24                   | 11                  | 76                   | 79                  |
| Average days                                                                            | 15.8 days           | 22.5 days            | 16.2 days           | 19.8 days           | 20.7 days            | 11.1 days           | 9.3 days            | 3.5 days             | 5.4 days            | 12.6 Days            | 14.3 Days           |

|                                                              |                     |                      |                     |                     |                      |                     |                     |                      |                     |                      |                     |
|--------------------------------------------------------------|---------------------|----------------------|---------------------|---------------------|----------------------|---------------------|---------------------|----------------------|---------------------|----------------------|---------------------|
| current individuals have been waiting                        |                     |                      |                     |                     |                      |                     |                     |                      |                     |                      |                     |
| Range of Days on waitlist                                    | 2-23 days           | 3-44 days            | 2-28 days           | 3-34 days           | 1-36 days            | 1-18 days           | 1-17 days           | 1-9 days             | 3-10 days           | 1-28 days            | 1-28 days           |
| <b>2. Regarding individuals found GEI and ordered to OSH</b> |                     |                      |                     |                     |                      |                     |                     |                      |                     |                      |                     |
|                                                              | <i>As of 1/5/22</i> | <i>As of 1/28/22</i> | <i>As of 5/1/22</i> | <i>As of 9/1/22</i> | <i>As of 12/1/22</i> | <i>As of 4/1/23</i> | <i>As of 7/1/23</i> | <i>As of 11/1/23</i> | <i>As of 4/1/24</i> | <i>As of 11/1/24</i> | <i>As of 4/1/25</i> |
| Total number of individuals                                  | 15                  | 4                    | 3                   | 4                   | 0                    | 1                   | 1                   | 1                    | 0                   | 2                    | 2                   |
| Average days waiting                                         | 45.6 days           | 23 days              | 18 days             | 13.0 days           | N/A                  | 26.0 days           | 10.0 days           | 1.0 days             | N/A                 | 13.0 days            | 12.0 days           |
| Range of Days on waitlist                                    | 1-110 days          | 17-28 days           | 12-26 days          | 3-20 days           | N/A                  | 26 days             | 10 days             | 1 day                | N/A                 | 9-17 days            | 12 days             |

\*The marked increase in numbers awaiting admission was most likely a residual of the pauses in admissions due to COVID-19

**Table 2. Updated Waitlist Numbers April 2025**

| .370's    | # on list | Average days waiting | GEI's    | # on list | Average days waiting |
|-----------|-----------|----------------------|----------|-----------|----------------------|
| 4/29/2025 | 36        | 18                   | April    | 6         | 18                   |
| 4/22/2025 | 56        | 19.9                 | March    | 2         | 25.5                 |
| 4/15/2025 | 60        | 21.5                 | February | 3         | 26.6                 |
| 4/8/2025  | 57        | 22                   | January  | 2         | 30                   |

**Table 3** and **Table 4** show the capacity and census at OSH, respectively. As I have noted earlier, the hospital uses an operational active capacity metric and has not shown any significant changes in this number since my prior report. In other words, the licensed capacity

reflects the number of actual physical “beds”, but the active capacity reflects those beds that are “usable” considering staffing levels, needed repair work, or the designation of the bed for specific types of patients (such as those with higher medical need, or female patients). Those latter beds are available but have been unfilled for long periods due to a lack of an appropriate candidate who is next on the admission list. During this reporting period I discussed with OHA and OSH leadership that the hospital had not had a census close to the available 705 beds for quite some time. I have addressed with the hospital ways to adjust its admissions protocols to ensure that beds are used up to the active capacity. Based on these conversations the hospital has re-done its admissions procedures to ensure that all 705 beds are filled at any given time.

**Table 3: OSH Bed Capacities as of 4/1/25**

| Site                           | Licensed Capacity | Active Capacity |
|--------------------------------|-------------------|-----------------|
| Salem Main Campus HLOC         | 502               | 474             |
| Salem Main Campus SRTF         | 90                | 87              |
| <b>Salem Main Campus Total</b> | <b>592</b>        | <b>561</b>      |
| Junction City HLOC             | 75                | 72              |
| Junction City SRTF             | 75                | 72              |
| <b>Junction City Total</b>     | <b>150</b>        | <b>144</b>      |
| <b>OSH Total</b>               | <b>742</b>        | <b>705</b>      |

**Table 4. OSH Census as of 4/1/25**

| Date      | Aid & Assist | PSRB | Civil Commitment | Other | Total |
|-----------|--------------|------|------------------|-------|-------|
| 9/1/2022  | 410          | 275  | 14               | 1     | 700   |
| 12/1/2022 | 396          | 279  | 13               | 0     | 688   |
| 4/1/2023  | 400          | 279  | 11               | 1     | 691   |
| 7/1/2023  | 389          | 281  | 13               | 1     | 684   |

|           |     |     |    |   |     |
|-----------|-----|-----|----|---|-----|
| 11/1/2023 | 366 | 291 | 17 | 1 | 675 |
| 4/1/2024  | 360 | 288 | 30 | 0 | 678 |
| 11/1/2024 | 375 | 270 | 27 | 8 | 680 |
| 4/1/2025  | 383 | 269 | 28 | 3 | 683 |

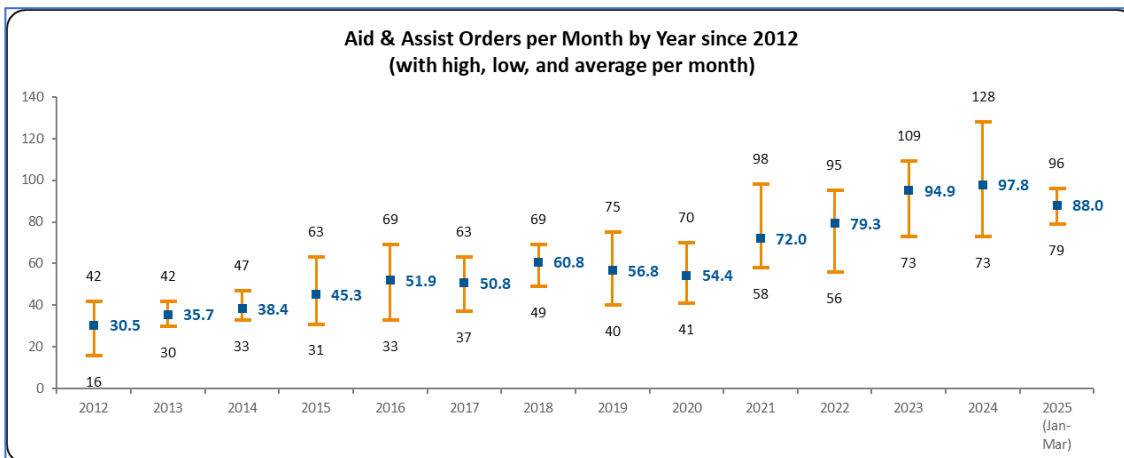
Since my last report in November 2024, December had the highest orders in a month, totaling 105. (See **Table 5** and **Figure 2**). There were 98 orders also received in April 2025 not reflected in the chart below. The overall trend of Aid and Assist Orders has been increasing, but for one month over another there may be no significant change, except that the first quarter of 2025 does appear slightly lower than the prior year. GEI admissions have fallen between one and nine per month over the last several years.

**Table 5. Aid and Assist and GEI Orders**

| Number of Orders Received | Aid & Assist | GEI                           |
|---------------------------|--------------|-------------------------------|
| April 2022                | 79           | 7 (4 standard / 3 revocation) |
| May 2022                  | 76           | 7 (4 standard / 3 revocation) |
| June 2022                 | 73           | 6 (4 standard / 2 revocation) |
| July 2022                 | 72           | 5 (3 standard / 2 revocation) |
| August 2022               | 81           | 7 (4 standard / 3 revocation) |
| September 2022            | 84           | 6 (5 standard / 1 revocation) |
| October 2022              | 95           | 3 (3 standard / 0 revocation) |
| November 2022             | 89           | 6 (2 standard / 4 revocation) |
| December 2022             | 74           | 4 (4 standard / 0 revocation) |
| January 2023              | 107          | 3 (3 standard / 0 revocation) |
| February 2023             | 78           | 5 (3 standard / 2 revocation) |
| March 2023                | 109          | 7 (2 standard / 5 revocation) |
| April 2023                | 103          | 5 (2 standard / 3 revocation) |
| May 2023                  | 94           | 7 (3 standard / 4 revocation) |
| June 2023                 | 86           | 1 (1 standard / 0 revocation) |
| July 2023                 | 73           | 3 (0 standard / 3 revocation) |
| August 2023               | 109          | 5 (3 standard / 2 revocation) |
| September 2023            | 93           | 7 (6 standard / 1 revocation) |
| October 2023              | 97           | 3 (2 standard / 1 revocation) |
| November 2023             | 98           | 3 (2 standard / 1 revocation) |
| December 2023             | 92           | 3 (2 standard / 1 revocation) |
| January 2024              | 83           | 4 (4 standard / 0 revocation) |

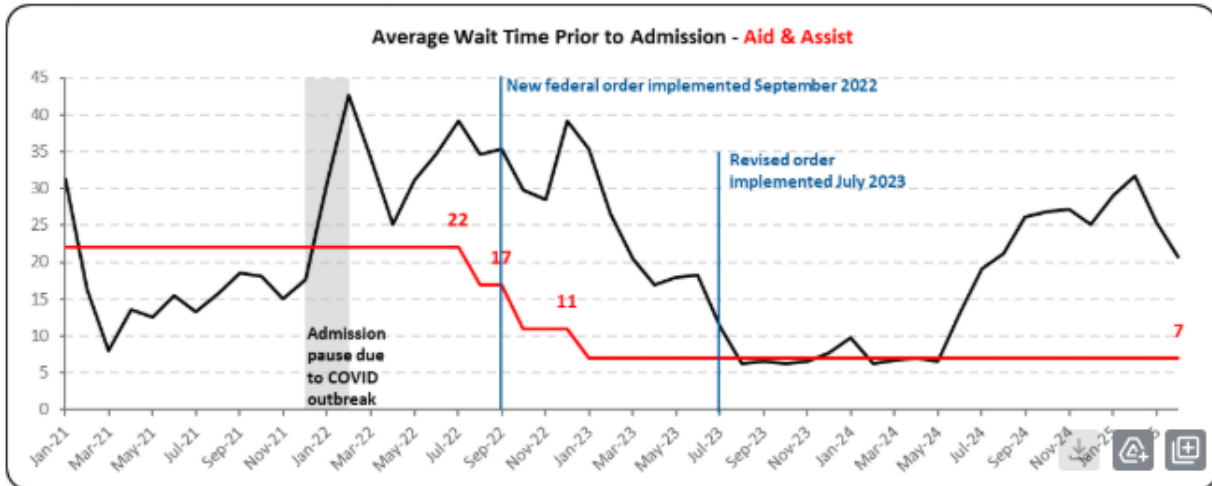
|                |     |                               |
|----------------|-----|-------------------------------|
| February 2024  | 73  | 9 (3 standard / 6 revocation) |
| March 2024     | 87  | 2 (2 standard / 0 revocation) |
| April 2024     | 99  | 1 (1 standard / 0 revocation) |
| May 2024       | 127 | 7 (3 standard / 4 revocation) |
| June 2024      | 90  | 2 (0 standard / 2 revocation) |
| July 2024      | 128 | 3 (0 standard / 3 revocation) |
| August 2024    | 99  | 4 (3 standard / 1 revocation) |
| September 2024 | 92  | 6 (6 standard / 0 revocation) |
| October 2024   | 102 | 6 (3 standard / 3 revocation) |
| November 2024  | 89  | 5 (3 standard / 2 revocation) |
| December 2024  | 105 | 3 (3 standard / 0 revocation) |
| January 2025   | 89  | 2 (2 standard / 0 revocation) |
| February 2025  | 79  | 4 (3 standard / 1 revocation) |
| March 2025     | 96  | 8 (4 standard / 4 revocation) |

**Figure 2. Aid & Assist Admissions/Orders Trends through March 2025**



**Figure 3** shows progress toward benchmarks set forth in my June 2022 report and more recent compliance with the overarching 7-day admission time as updated through the end of April 2025. For this reporting period, the trend line showed movement away from compliance but is now moving in the direction toward compliance. In April 2025, OSH admitted 128 A&A patients (tied for the most ever in a month) with an average wait time of 20.7 days (down from 25.4 days in March).

**Figure 3. Admission Wait Time Projections Compared to Benchmarks Set in Second Report based on data as of 5/1/25**



**Table 6** below shows data related to the September 2022 order by Judge Mosman. Of the 3044 individuals who were admitted to OSH after the 9/1/22 order (so-called “Cohort 2”) and through 5/1/25, 1123 were opined able, and 167 were opined never able. There were 585 individuals discharged to community restoration, 132 whose charges were dismissed or were released, 600 who were discharged after receiving a restoration limit notice, and four were discharged at the end of statutory jurisdiction. Thus, most people who had not been restored but had come to the end of their allowable restoration period appear to be discharged to community restoration. **Table 7** shows the type of findings in the month of March with most defendants found able, and about half as many discharged to community restoration.

**Table 6. Discharge Data Related to the 9/1/22 Order by Judge Mosman**

- **Cohort 1:** Patients at OSH at the time of the Federal Court Order
- **Cohort 2:** Patients admitted to OSH after the issuance of the Federal Court Order on 9/1/22

**OSH Restoration Limit Report**  
(data are current as of 05/01/2025)

| Cohort 1       | At OSH<br>as of<br>9/1/2022 | At OSH<br>as of<br>5/3/2025 | Restoration Limit Notice Outcomes<br>(total since 9/1/2022) |                                                           |                                                        | Discharge Reasons<br>(total since 9/1/2022) |                     |                          |                                     |                                                        |                                     |          |
|----------------|-----------------------------|-----------------------------|-------------------------------------------------------------|-----------------------------------------------------------|--------------------------------------------------------|---------------------------------------------|---------------------|--------------------------|-------------------------------------|--------------------------------------------------------|-------------------------------------|----------|
|                |                             |                             | 30-Day RL<br>Notices Sent                                   | Discharged Prior to<br>Meeting 30-Day<br>RL Notice Period | Discharged After<br>Meeting 30-Day<br>RL Notice Period | Found<br>Able                               | Found<br>Never Able | Community<br>Restoration | Charges<br>Dismissed<br>or Released | Discharged After<br>Meeting 30-Day<br>RL Notice Period | End of<br>Statutory<br>Jurisdiction | Other    |
|                |                             |                             |                                                             |                                                           |                                                        |                                             |                     |                          |                                     |                                                        |                                     |          |
| Misdemeanor    | 85                          | 0                           | 51                                                          | 25                                                        | 26                                                     | 18                                          | 2                   | 29                       | 7                                   | 26                                                     | 3                                   |          |
| Felony         | 217                         | 0                           | 101                                                         | 31                                                        | 70                                                     | 68                                          | 13                  | 57                       | 9                                   | 70                                                     |                                     |          |
| Violent Felony | 107                         | 1                           | 43                                                          | 25                                                        | 17                                                     | 45                                          | 29                  | 6                        | 3                                   | 17                                                     | 2                                   | 4        |
| <b>Total</b>   | <b>409</b>                  | <b>1</b>                    | <b>195</b>                                                  | <b>81</b>                                                 | <b>113</b>                                             | <b>131</b>                                  | <b>44</b>           | <b>92</b>                | <b>19</b>                           | <b>113</b>                                             | <b>5</b>                            | <b>4</b> |

| Cohort 2       | Admitted<br>since<br>9/1/2022 | At OSH<br>as of<br>5/3/2025 | Restoration Limit Notice Outcomes<br>(total since 9/1/2022) |                                                           |                                                        | Discharge Reasons<br>(total since 9/1/2022) |                     |                          |                                     |                                                        |                                     |           |
|----------------|-------------------------------|-----------------------------|-------------------------------------------------------------|-----------------------------------------------------------|--------------------------------------------------------|---------------------------------------------|---------------------|--------------------------|-------------------------------------|--------------------------------------------------------|-------------------------------------|-----------|
|                |                               |                             | 30-Day RL<br>Notices Sent                                   | Discharged Prior to<br>Meeting 30-Day<br>RL Notice Period | Discharged After<br>Meeting 30-Day<br>RL Notice Period | Found<br>Able                               | Found<br>Never Able | Community<br>Restoration | Charges<br>Dismissed<br>or Released | Discharged After<br>Meeting 30-Day<br>RL Notice Period | End of<br>Statutory<br>Jurisdiction | Other     |
|                |                               |                             |                                                             |                                                           |                                                        |                                             |                     |                          |                                     |                                                        |                                     |           |
| Misdemeanor    | 986                           | 93                          | 881                                                         | 488                                                       | 333                                                    | 227                                         | 37                  | 229                      | 60                                  | 333                                                    | 3                                   | 4         |
| Felony         | 1619                          | 230                         | 654                                                         | 362                                                       | 247                                                    | 642                                         | 90                  | 338                      | 67                                  | 247                                                    |                                     | 5         |
| Violent Felony | 439                           | 93                          | 112                                                         | 71                                                        | 20                                                     | 254                                         | 40                  | 18                       | 5                                   | 20                                                     | 1                                   | 8         |
| <b>Total</b>   | <b>3044</b>                   | <b>416</b>                  | <b>1647</b>                                                 | <b>921</b>                                                | <b>600</b>                                             | <b>1123</b>                                 | <b>167</b>          | <b>585</b>               | <b>132</b>                          | <b>600</b>                                             | <b>4</b>                            | <b>17</b> |

**Table 7. Legal Status of AA Discharges in March 2025 based on Hospital Data**

**March 2025 A&A Discharges**

| Reason                        | Cohort 1 | Cohort 2  | Total     |
|-------------------------------|----------|-----------|-----------|
| Able                          |          | 42        | 42        |
| Never Able                    |          | 6         | 6         |
| Med Never                     |          | 1         | 1         |
| Community Restoration         |          | 26        | 26        |
| Dismissed                     |          | 1         | 1         |
| End of Statutory Jurisdiction |          |           | 0         |
| Other                         |          | 8         | 8         |
| Restoration Limit             |          | 4         | 4         |
| <b>Total</b>                  | <b>0</b> | <b>88</b> | <b>88</b> |

The plaintiffs had asserted that the defendants had not anticipated predictable increases in orders, but the chart below shows that there were predictions of increases, and that the order numbers have at times exceeded predictions (see **Table 8**).

**Table 8. Projections vs. Actuals Admissions, Discharges, and New Restoration Orders**

| Month  | Original Projections (Pre-Federal Order Implementation) |            |            |            | Actuals (with Current Projections) |            |            |            |
|--------|---------------------------------------------------------|------------|------------|------------|------------------------------------|------------|------------|------------|
|        | Discharges                                              | Admissions | New Orders | Admit List | Discharges                         | Admissions | New Orders | Admit List |
| Sep-22 | 67                                                      | 67         | 74         | 77         | 85                                 | 76         | 84         | 86         |
| Oct-22 | 90                                                      | 90         | 74         | 61         | 90                                 | 91         | 95         | 90         |
| Nov-22 | 90                                                      | 90         | 74         | 45         | 85                                 | 81         | 89         | 104        |
| Dec-22 | 95                                                      | 95         | 74         | 24         | 92                                 | 77         | 74         | 90         |
| Jan-23 | 97                                                      | 97         | 74         | 10         | 93                                 | 101        | 107        | 98         |
| Feb-23 | 97                                                      | 97         | 74         | 10         | 94                                 | 107        | 78         | 70         |
| Mar-23 | 107                                                     | 107        | 79         | 10         | 129                                | 128        | 109        | 51         |
| Apr-23 | 89                                                      | 89         | 79         | 10         | 108                                | 107        | 103        | 46         |
| May-23 | 89                                                      | 89         | 79         | 10         | 88                                 | 87         | 94         | 57         |
| Jun-23 | 89                                                      | 89         | 79         | 10         | 101                                | 97         | 86         | 42         |
| Jul-23 | 87                                                      | 87         | 79         | 10         | 103                                | 104        | 73         | 14         |
| Aug-23 | 87                                                      | 87         | 79         | 10         | 112                                | 100        | 109        | 17         |
| Sep-23 | 90                                                      | 90         | 84         | 10         | 102                                | 95         | 93         | 19         |
| Oct-23 | 91                                                      | 91         | 84         | 10         | 97                                 | 93         | 97         | 24         |
| Nov-23 | 91                                                      | 91         | 84         | 10         | 103                                | 108        | 98         | 14         |
| Dec-23 | 92                                                      | 92         | 84         | 10         | 64                                 | 83         | 92         | 23         |
| Jan-24 | 92                                                      | 92         | 84         | 10         | 96                                 | 82         | 83         | 22         |
| Feb-24 | 92                                                      | 92         | 84         | 10         | 97                                 | 81         | 73         | 14         |
| Mar-24 | 92                                                      | 92         | 89         | 10         | 79                                 | 85         | 87         | 11         |
| Apr-24 | 92                                                      | 92         | 89         | 10         | 84                                 | 96         | 99         | 22         |
| May-24 | 92                                                      | 92         | 89         | 10         | 93                                 | 108        | 127        | 34         |
| Jun-24 | 92                                                      | 92         | 89         | 10         | 79                                 | 74         | 90         | 58         |
| Jul-24 | 92                                                      | 92         | 89         | 10         | 102                                | 103        | 128        | 79         |
| Aug-24 | 92                                                      | 92         | 89         | 10         | 95                                 | 95         | 99         | 79         |
| Sep-24 | 92                                                      | 92         | 94         | 10         | 86                                 | 84         | 92         | 85         |
| Oct-24 | 92                                                      | 92         | 94         | 10         | 109                                | 107        | 102        | 76         |
| Nov-24 | 92                                                      | 92         | 94         | 10         | 80                                 | 77         | 89         | 91         |
| Dec-24 | 92                                                      | 92         | 94         | 10         | 99                                 | 111        | 105        | 82         |
| Jan-25 | 92                                                      | 92         | 97         | 10         | 93                                 | 93         | 89         | 84         |
| Feb-25 | 92                                                      | 92         | 97         | 10         | 90                                 | 97         | 79         | 68         |
| Mar-25 | 92                                                      | 92         | 97         | 10         | 88                                 | 85         | 96         | 79         |
| Apr-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 84         |
| May-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 89         |
| Jun-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 94         |
| Jul-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 99         |
| Aug-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 104        |
| Sep-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 109        |
| Oct-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 114        |
| Nov-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 119        |
| Dec-25 | 92                                                      | 92         | 97         | 10         | 92                                 | 92         | 97         | 124        |

Community restoration data is currently undergoing some improvements as recommended in my earlier reports. Current community restoration data that is reported is based on what the CMHPs report on a quarterly basis. The data is depicted in **Table 9** and **Table 10**, showing that community restoration episodes for the 2024 numbered 1165, up from 1007 in 2023, and much higher than 671 in 2022. In 2024, 313 people had community restoration episodes for over one year (1162-849). Because elements of the data lack specificity and given that it is hand collected in many ways, the data is not as readily conclusive as hospital restoration data. Since

community restoration began, about 500 community restoration episodes lasted over one year (2687-2181). The mean number of days for community restoration was 223, with a maximum number of days as 1661, or four and a half years.

**Table 9. CMHP Reported Completed Community Restoration Data 1/1/2019-12/31/2024**

**CMHP Reported Community Restoration Data 1/1/2019-12/31/2024\*\***

|                                     |                                     |                                           |
|-------------------------------------|-------------------------------------|-------------------------------------------|
| # of Community Restoration Episodes | 2696                                |                                           |
| # of Days Minimum                   | 1                                   |                                           |
| # of Days Maximum                   | 1661                                |                                           |
| # of Days Mean                      | 223                                 |                                           |
| # of Days Median                    | 165                                 |                                           |
| Days in Community Restoration       | # of Community Restoration Episodes | % of Total Community Restoration Episodes |
| 0-90                                | 799                                 | 29.64%                                    |
| 0-180                               | 1437                                | 53.30%                                    |
| 0-365                               | 2181                                | 80.90%                                    |
| 0-730                               | 2633                                | 97.66%                                    |
| 0-1095                              | 2687                                | 99.67%                                    |

- This chart includes all community restoration cases, completed and ongoing.
- The reported numbers may change in the near future as counties are being asked to review and clean their data as necessary.
- Curry cases are excluded because no report was submitted in CY2023 and CY2024.

**Table 10. CMHP Reported Year by Year Data 1/1/2019 -12/31/2024**

| CMHP Reported Community Restoration Data 1/1/2019-12/31/2024                                                                         |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |  |
|--------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------|-------------------------------------------|-------------------------------------|-------------------------------------------|-------------------------------------|-------------------------------------------|-------------------------------------|-------------------------------------------|-------------------------------------|-------------------------------------------|-------------------------------------|-------------------------------------------|--|
|                                                                                                                                      | 2019                                |                                           | 2020                                |                                           | 2021                                |                                           | 2022                                |                                           | 2023                                |                                           | 2024                                |                                           |  |
| # of Community Restoration Episodes                                                                                                  | 321                                 |                                           | 370                                 |                                           | 441                                 |                                           | 621                                 |                                           | 1007                                |                                           | 1165                                |                                           |  |
| # of Days Minimum                                                                                                                    | 2                                   |                                           | 1                                   |                                           | 1                                   |                                           | 1                                   |                                           | 1                                   |                                           | 1                                   |                                           |  |
| # of Days Maximum                                                                                                                    | 1661                                |                                           | 1661                                |                                           | 1661                                |                                           | 1661                                |                                           | 1661                                |                                           | 1636                                |                                           |  |
| # of Days Mean                                                                                                                       | 285                                 |                                           | 313                                 |                                           | 286                                 |                                           | 290                                 |                                           | 305                                 |                                           | 261                                 |                                           |  |
| # of Days Median                                                                                                                     | 205                                 |                                           | 224                                 |                                           | 196                                 |                                           | 196                                 |                                           | 246                                 |                                           | 212                                 |                                           |  |
| Days in Community Restoration                                                                                                        | # of Community Restoration Episodes | % of Total Community Restoration Episodes | # of Community Restoration Episodes | % of Total Community Restoration Episodes | # of Community Restoration Episodes | % of Total Community Restoration Episodes | # of Community Restoration Episodes | % of Total Community Restoration Episodes | # of Community Restoration Episodes | % of Total Community Restoration Episodes | # of Community Restoration Episodes | % of Total Community Restoration Episodes |  |
| 0-90                                                                                                                                 | 66                                  | 20.56%                                    | 60                                  | 16.22%                                    | 99                                  | 22.45%                                    | 127                                 | 20.45%                                    | 194                                 | 19.27%                                    | 312                                 | 26.78%                                    |  |
| 0-180                                                                                                                                | 144                                 | 44.86%                                    | 150                                 | 40.54%                                    | 197                                 | 44.67%                                    | 280                                 | 45.09%                                    | 396                                 | 39.32%                                    | 506                                 | 43.43%                                    |  |
| 0-365                                                                                                                                | 233                                 | 72.59%                                    | 256                                 | 69.19%                                    | 325                                 | 73.70%                                    | 464                                 | 74.72%                                    | 637                                 | 63.26%                                    | 849                                 | 72.88%                                    |  |
| 0-730                                                                                                                                | 303                                 | 94.39%                                    | 343                                 | 92.70%                                    | 414                                 | 93.88%                                    | 567                                 | 91.30%                                    | 963                                 | 95.63%                                    | 1130                                | 97.00%                                    |  |
| 0-1095                                                                                                                               | 316                                 | 98.44%                                    | 362                                 | 97.84%                                    | 432                                 | 97.96%                                    | 612                                 | 98.55%                                    | 1002                                | 99.50%                                    | 1162                                | 99.74%                                    |  |
| - This chart includes all community restoration cases, completed and ongoing.                                                        |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |  |
| - The reported numbers may change in the near future as counties are being asked to review and clean the reported data as necessary. |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |  |
| - Curry cases are excluded because no report was submitted in CY2023 and CY2024.                                                     |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |                                     |                                           |  |

Forensic Evaluation Services (FES) data continues to show high numbers of evaluations conducted by FES staff, including requests for evaluations of individuals outside of OSH. **Table 11** shows recent data on active cases for which FES has been assigned to evaluate, 446 of which are not currently at OSH. **Table 12** depicts the types of evaluations being conducted by FES each month. **Figure 4** is a graphic depiction of **Table 12**. **Figure 5** shows the number of FES evaluations of people in community restoration. Those numbers have increased significantly since the valve was opened for these to be conducted sooner.

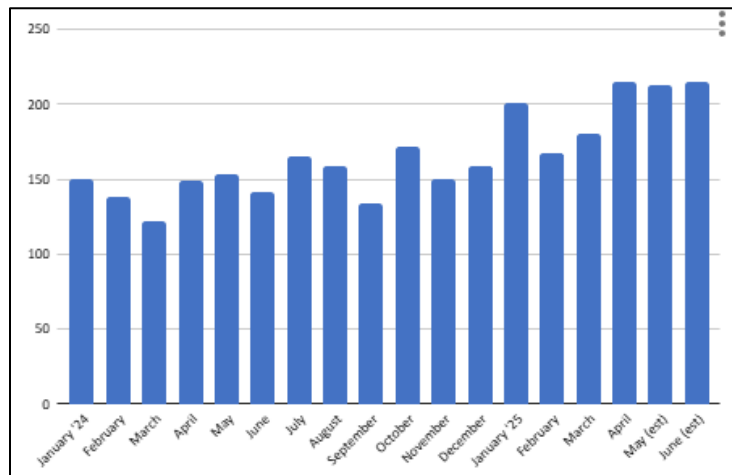
**Table 11. Number of Active FES Cases as of 4/1/25**

| Type of Evaluation and Location | Number     |
|---------------------------------|------------|
| .370 Evaluations at OSH         | 374        |
| .370 Evaluations not at OSH     | 357        |
| .365 Evaluations not at OSH     | 66         |
| .315 Evaluations not at OSH     | 23         |
| <b>Total Cases</b>              | <b>820</b> |

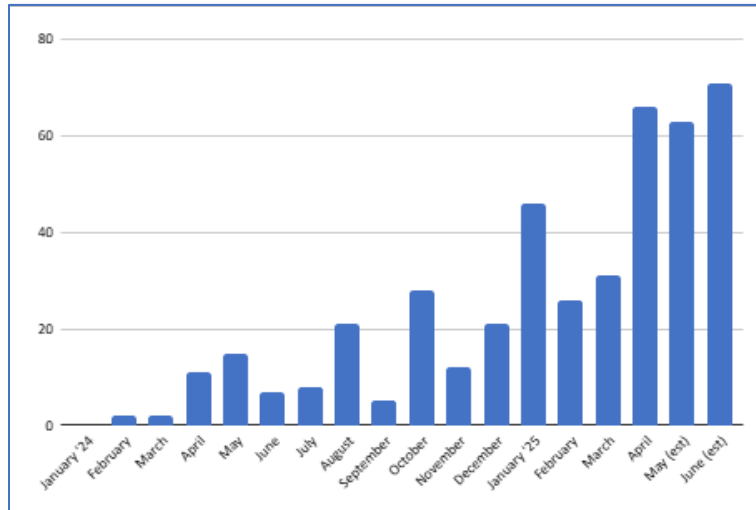
**Table 12. Number of FES Evaluations from Jan 2024 to those Scheduled in June 2025**

| Month             | Number of interviews | Inpatient 370s | 365s      | 315/135  | Outpatient 370s |
|-------------------|----------------------|----------------|-----------|----------|-----------------|
| January '24       | 150                  | 144            | 4         | 2        | 0               |
| February          | 138                  | 121            | 12        | 3        | 2               |
| March             | 122                  | 108            | 7         | 5        | 2               |
| April             | 149                  | 129            | 8         | 1        | 11              |
| May               | 153                  | 125            | 11        | 2        | 15              |
| June              | 142                  | 121            | 14        | 0        | 7               |
| July              | 165                  | 148            | 8         | 1        | 8               |
| August            | 159                  | 127            | 8         | 3        | 21              |
| September         | 134                  | 117            | 9         | 3        | 5               |
| October           | 172                  | 124            | 17        | 3        | 28              |
| November          | 150                  | 121            | 15        | 2        | 12              |
| December          | 159                  | 125            | 11        | 2        | 21              |
| January '25       | 201                  | 141            | 12        | 2        | 46              |
| February          | 168                  | 125            | 14        | 3        | 26              |
| March             | 180                  | 134            | 14        | 1        | 31              |
| April             | 215                  | 127            | 20        | 2        | 66              |
| May (est)         | 213                  | 131            | 14        | 5        | 63              |
| <b>June (est)</b> | <b>215</b>           | <b>127</b>     | <b>14</b> | <b>3</b> | <b>71</b>       |

**Figure 4: Graphic Depiction of Number of FES Evaluations from Jan 2024 to those Scheduled in June 2025**



**Figure 5: Graphic Depiction of Number of FES Community Restoration Evaluations from Jan 2024 to those Scheduled in June 2025**



The data in **Table 13** are constructed slightly differently than similar data reported in my prior reports. Nevertheless, this data shows that approximately one third of people with lower-level offenses who are admitted to OSH do not need a Hospital Level of Care within 10 days of discharge. This is down from nearly 40% of people who fell into this same category (perhaps explained by some of the increased utilization of community restoration) but still is a significant group that warrants further study.

**Table 13. 10-Day RTP Assessment Outcomes (August 2022 through March 2025)**

| Period         | 10-Day RTP Assessments | Patients Found RTP | Percent Found RTP |
|----------------|------------------------|--------------------|-------------------|
| 2022 (Aug-Dec) | 205                    | 87                 | 42.4%             |
| 2023           | 648                    | 250                | 38.6%             |
| 2024           | 596                    | 170                | 28.5%             |
| 2025 (Jan-Mar) | 168                    | 48                 | 28.6%             |
| <b>Total</b>   | <b>1617</b>            | <b>555</b>         | <b>34.3%</b>      |

The mediation that resulted in the Second Amended order from 7/3/23 offered exceptions to the original restoration duration limits. The information in **Table 14** shows how many of these extension requests were received and granted.

**Table 14. Number of 30-day and 180-day Requests to Extend Restoration Duration**

| Period              | 30-Day Extension Requests |                 |                 |            | 180-Day Extension Requests |                 |                 |           |
|---------------------|---------------------------|-----------------|-----------------|------------|----------------------------|-----------------|-----------------|-----------|
|                     | Initial                   | 2 <sup>nd</sup> | 3 <sup>rd</sup> | Total      | Initial                    | 2 <sup>nd</sup> | 3 <sup>rd</sup> | Total     |
| Jul 2023 – Dec 2023 | 17                        |                 |                 | 17         | 17                         | 5               |                 | 22        |
| 2024                | 74                        | 5               | 1               | 80         | 42                         | 11              | 4               | 57        |
| 2025 YTD (Jan-Mar)  | 24                        |                 |                 | 24         |                            | 4               | 3               | 7         |
| <b>Total</b>        | <b>115</b>                | <b>5</b>        | <b>1</b>        | <b>121</b> | <b>59</b>                  | <b>20</b>       | <b>7</b>        | <b>86</b> |

*Currently all competency restoration extension requests are being granted, whether they meet criteria or not.*

Civil expedited admission requests and admissions were also examined. The data produced by OSH indicated is in **Table 15**. This data shows that about 59% of these requests have been accepted overall, with 53% accepted in the first three months of 2025.

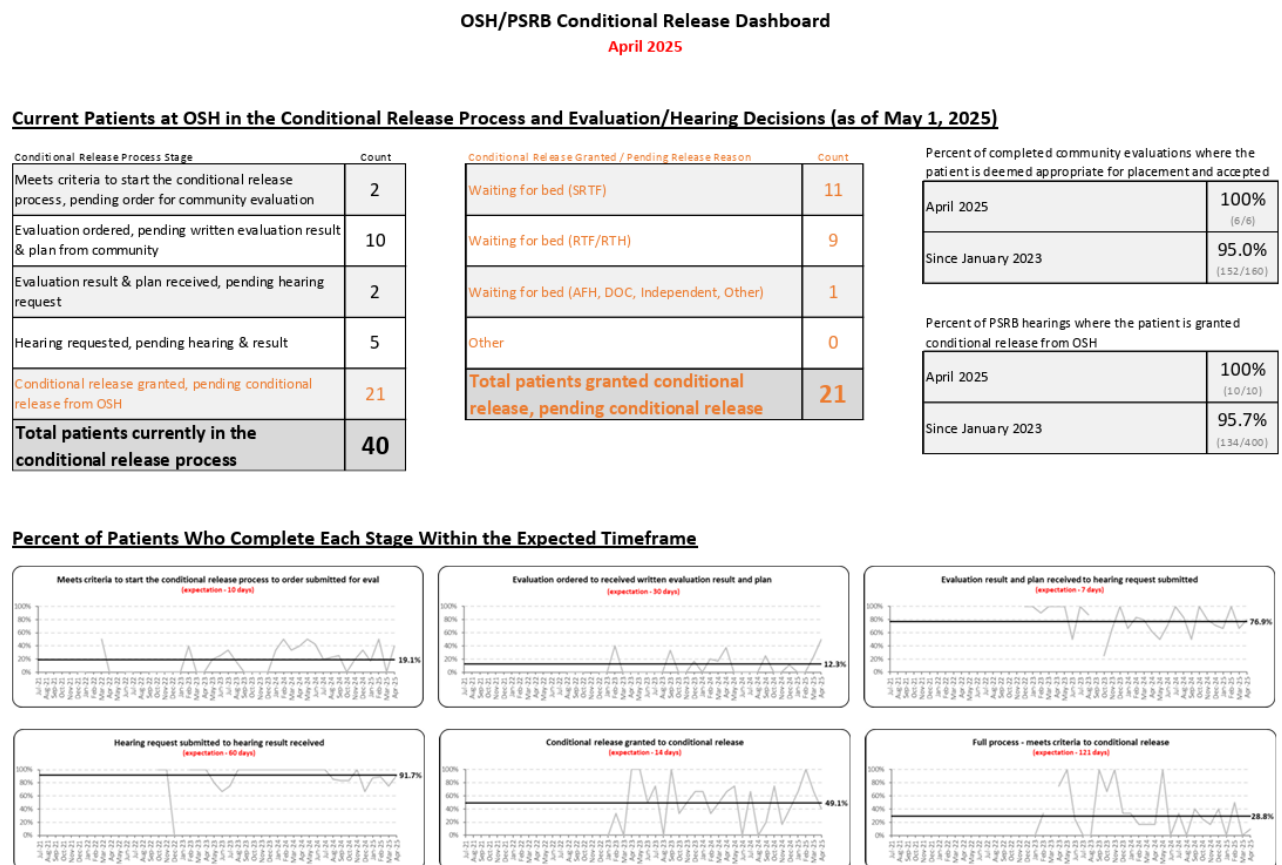
**Table 15. Civil Expedited Admission Requests**

| Period              | Requests  | Accepted  | Denied    | Under Review |
|---------------------|-----------|-----------|-----------|--------------|
| Sep 2022 – Dec 2023 | 22        | 14        | 8         |              |
| 2024                | 58        | 34        | 24        |              |
| 2025 YTD (Jan-Mar)  | 17        | 9         | 7         | 1            |
| <b>Total</b>        | <b>97</b> | <b>57</b> | <b>39</b> | <b>1</b>     |

**Figure 6** depicts information that has been put together to track the efficiencies in GEI processes. It should be noted that about one month ago, three of the time periods were shortened per my earlier recommendations to reduce time whenever it was simply within the

control of OSH administrative processes. The total time expected from referral for evaluation to discharge is now 121 days instead of 171 days. Still, as can be seen, none of the metrics are at 100% compliance. For example, from the middle bottom box, only 49.1% of individuals are discharged timely from the time conditional release is granted to the time they are discharged, and 28.8% complete the full process to discharge within the required 121 allotted days. There is an upward improvement in time to get the order submitted for evaluation (top left box), but a downward trend in actual discharge metrics.

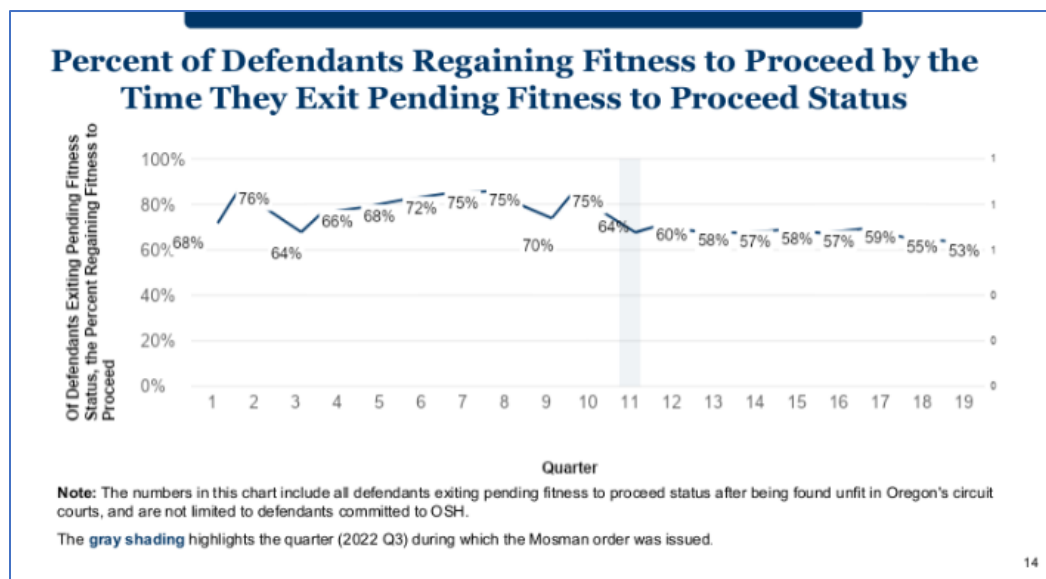
**Figure 6. Conditional Release and PSRB OSH Dashboard**



## Additional Data Points

Several additional data points are presented here for the Court’s review. For example, in a legislative workgroup examining timelines for restoration potential statutory change, OJD presented several data points. One data chart showed the percentage of defendants who had regained fitness to proceed as decreasing since the Mosman Order was issued. This is depicted in **Figure 7**. In a separate data report by OHA/OSH, it was reported that OSH saw only a 16% decrease in the number of patients receiving a dischargeable finding or moving into community restoration as a result of the restoration limits, and that the time limits under the federal order have created increased patient flow and increased the ability to admit individuals to OSH by about 30-35%.

**Figure 7. Percent of Defendants Regaining Fitness to Proceed by the Time they Exit Pending Fitness to Proceed Status**



In a separate data report from a legislative briefing produced by OJD and ODAA, 38 defendants from 2022 to 2023 in Marion County had dismissed cases, eight of which were thought to be unable and not likely to be restored. Of the remaining 30 defendants, 13 had new charges, 11

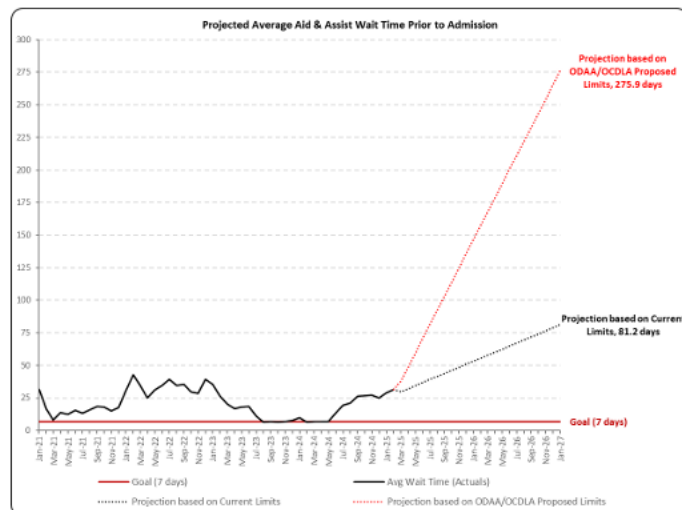
had returned to OSH, and two had active warrants for their arrest (see discussion below in my summary comments at the end of this report). Statewide data previously reported does support that often individuals in community restoration have charges dismissed.

Data presented by OHA during legislative hearings examined the two leading proposed statutes related to community restoration timelines. Figure 8 represents one of their projected data summaries, showing that the ODAA/OCDLA proposal would create a significant backwards trend away from compliance. With everything being status quo, the current timelines imposed by the federal court, as represented in the OHA/OSH legislative HB 3051 would also not achieve compliance alone. My recommendations from the very beginning have included many more elements that relate to building an expansive community system of care to support individuals and decrease the reliance about the criminal courts and Aid and Assist as a way into behavioral health services.

**Figure 8: OHA Projected Average Aid and Assist Wait Times for ODAA/OCDLA and OHA/OSH proposed legislation**

## ODAA/OCDLA Option Compared to HB 3051

- If implemented today, it is predicted that the ODAA/OCDLA restoration limit changes would, by the end of 2026, result in a wait list of 684 patients and an average wait time of 275 days.
- With the current rate of new orders at 97.8 per month, even the current restoration limits will, by the end of 2026, result in a wait list of 201 patients and an average wait time of 81.2 days.
- The restoration limits in HB 3051 are necessary, but not sufficient alone, to meet the goal of admission within 7 days.



I spoke with leadership at NWRRC on several occasions and learned that the census was not consistently being managed to full capacity. I then asked NWRRC leadership to provide a written summary about daily census including their daily data in April. The data and information they reported included:

- Bed average for February was 32.25 and the average for March was 31.03
- 57 aid & Assist residents have been at NWRRC for Community Restoration
- There have been 23 releases to date
- From 4/1/25 to 4/23/25 the census reached 30 for one day, there were six days it reached 29, 10 days it was at 28, and six days it was at 27
- There are cases where it is difficult to secure a discharge from restoration

In a report produced by Optumas for the legislature in December 2024, cost data was reported as seen in **Table 16**. Of note, in this report a large proportion of individuals served within the Aid and Assist population are reported as not eligible for Medicaid. The service payment is not described to clarify this further.

**Table 16. Cost data for Unique Individuals Served as Reported by the CMHPs**

Data Analysis and Evaluation

CBIZ Optumas

Table 5 – Unique Individuals Served as Reported by the CMHPs

| Service / Individual Classification            | CY22          | CY23          | First Six Months of CY24 |
|------------------------------------------------|---------------|---------------|--------------------------|
| <b>Aid &amp; Assist</b>                        |               |               |                          |
| Not Eligible for Medicaid                      | 415           | 517           | 412                      |
| Medicaid Eligible                              | 644           | 971           | 894                      |
| Unknown                                        | 120           | 157           | 168                      |
| <b>Total</b>                                   | <b>1,179</b>  | <b>1,645</b>  | <b>1,474</b>             |
| <b>Civil Commitment</b>                        |               |               |                          |
| Not Eligible for Medicaid                      | 684           | 606           | 332                      |
| Medicaid Eligible                              | 1,427         | 1,382         | 907                      |
| Unknown                                        | 972           | 999           | 776                      |
| <b>Total</b>                                   | <b>3,083</b>  | <b>2,987</b>  | <b>2,015</b>             |
| <b>Crisis (includes non-mobile and mobile)</b> |               |               |                          |
| Not Eligible for Medicaid                      | 7,383         | 8,241         | 4,963                    |
| Medicaid Eligible                              | 15,727        | 16,421        | 11,261                   |
| Unknown                                        | 11,202        | 11,780        | 10,968                   |
| <b>Total</b>                                   | <b>34,312</b> | <b>36,442</b> | <b>27,192</b>            |

Table Notes:

1. This table does not represent data for every CMHP and year period. In some cases, CMHPs did not provide a total of unique individuals; however, these omissions should not significantly impact the figures presented here.

DRO provided me with data tracking recommitments of people civilly committed in Multnomah County to demonstrate some of the apparent backlogs in SRTFs. Since the Spring of 2024 through January 2025, DRO identified 29 people who had been civilly committed to SRTFs, 24 of which were recommitted for a second 180 days, and 11 of whom had more than 3 recommitments, and one who had been continuously committed since October 2019.

### **Select Updates from OHA:**

Director Sejal Hathi is currently out on leave, and Kris Kautz has been serving as Interim OHA Director. She has regularly attended meetings with the Neutral Expert.

OHA leadership began setting up the Extended Care Management Unit (ECMU) by November 2024 starting with work in Multnomah, Lane and Washington Counties. Several OHA staff have been dedicated to this team. I met with the team weekly to solve problems and track their progress. Recently those meetings have been folded into the regular weekly OHA OSH check in with me. The ECMU team has reported anecdotes of cases where they have been helpful. I have reviewed regular data with them as they are attempting to track their effectiveness with discharges.

Two cases that were shared included one from Lane County where there had been a lack of coordinated follow up and only two referrals for discharge placements, and it appeared that “everyone is just waiting for a referral answer.” In that same case one of the placements had accepted the individual but had not alerted anyone. There was also a wait for a forensic evaluation. In another case from Multnomah County, the individual had been accepted to NWRRC but it was not clear where he was on the waitlist for that placement. There was also a need for a plan for his substance use disorder that no one seemed to be executing without the ECMU’s direction.

The ECMU has begun to include the Choice contractor in the discussions to help secure placements. In addition, there was a recognition that more referrals need to be made for each individual since it is hard to know what space would open first. It will be important to continue to see if the ECMU is providing “added value” for expediting discharges.

The defendants and I continue to discuss the status of increasing bed capacity per my 10<sup>th</sup> report recommendations. One program, Jackson House, opened and was able to admit several GEI patients from OSH. However, as noted recently by DRO, the public facing dashboard of bed capacity does not have consistent numbers across two different time points. This data needs to be clarified.

To improve the potential for discharge of forensic patients from OSH, and with the thought that many with the AA or GEI legal status had more difficulty being accepted for placement, OHA issued a temporary rule, OAR 309-035-0163, which prioritized the acceptance of individuals in the AA and GEI and civil commitment processes, effective 1/17/25. This rule received pushback from the private hospitals, who increasingly were concerned that other voluntary individuals would not have access to residential placements, thereby causing their discharges to slow down. This was raised at the contempt hearing. To date, there has been no data I have seen that further examines this issue, the State indicated it would track this.

I received a Draft Community Restoration Annual Report and Draft Community Restoration Manual in February 2025 and have provided feedback. Although the Draft Community Restoration Manual had a great deal of rich information, it requires substantial edits to align with my original recommendations. The Draft Community Restoration Annual Report was largely a re-issuance of the data produced in the PDES report. I provided feedback to the state that the report should contain more current data, even if limited, regarding Community Restoration. We discussed the need for greater clarity about what data will be collected and reported in the next annual report. I was told that the data use agreements with OJD were being finalized, which would allow for more robust data sharing.

I discussed opportunities for jail diversion and OSH diversion at length with OHA and OSH and DOJ. Specifically regarding my earlier recommendation “1.B.7.b & 1.B.7.c.: OHA will monitor the OSH waitlist weekly. If the waitlist exceeds 10 days, OHA will initiate jail diversion meetings with CMHP to review current symptoms and explore appropriate alternative community restoration services, if available.” According to an analysis produced by the defendants, “Since June 12, 2024, none of the 897 individuals who were on the OSH waitlist were diverted because of OHA’s intervention. Only 6 cases were diverted from OSH based on actions other than OHA intervention.” After further discussion it was concluded that with there are several barriers to implementing meaningful actions after an order for OSH restoration is received that would divert individuals from the hospital. Barriers include status of the criminal matter and who has standing to “divert,” as well as timing issues, jail participation in in-reach, and timeliness to access evaluators and placements prior to being admitted to OSH from the waitlist.

OHA delivered initial training to defense attorneys, prosecutors and judges to help explain the behavioral health system options vis-a-vis the Aid and Assist and GEI populations. The trainings included information on what services are funded and how they are funded through the County Financial Assistance Agreements (CFAAs), differences between CMHP consultation and competency to stand trial evaluations, FES opinions, and the distinctions between competency restoration and qualifications for medically necessary services under Medicaid rules, to name a few. In addition, the training was designed to explain the use of the LOCUS for placement recommendations. The OHA staff conducting the trainings informed me that they had a great deal of pushback from judges who declined to participate, and they were having difficulty gathering other willing parties to attend the training. I have asked for feedback from the trainings that were offered as well as updated versions of the training after incorporating feedback I provided.

In conversations with OHA leadership, despite my having made this recommendation over two years ago, it appears there are still barriers to the work between OHA and the ODDS and APD

leadership to help facilitate work for AA populations with support needs for their intellectual and developmental disabilities, traumatic brain injuries and other cognitive disorders.

I have met several times with the Medicaid team and have brought on the expertise of Kirsten Beronio, JD. In these meetings we have discussed strategies for rule changes and contractual requirements to enhance care coordination. Two letters, one by Director Hathi and one by Interim OHA Director Kris Kautz, speak to requests and plans to delay the Request for Application that was scheduled for release in the summer of 2025, thereby extending contracts with CCOs through the end of December 2028. These delays raise concerns about how long potential reforms will take.

Regarding specific initiatives in Medicaid, I have discussed with Medicaid leadership the 1115 demonstration waiver that is due to help provide in reach for up to 90 days for individuals being released from jails and prisons. There is a great deal of planning taking place in this space and implementation is likely- but not soon enough to affect compliance with the Mink order for some time. We have also discussed other waiver strategies to fund services including the 1915i and 1915k waivers (that provide, respectively) additional services for people with serious mental illness and intellectual/developmental disability. Part of the issue for people in the AA process is that they are dropped from CCO rosters when they are institutionalized, and Medicaid needs to be re-instated in a meaningful way upon return to the community. This is not unique to Oregon, but this phenomenon requires problem solving. Oregon has had a strong array of certified community behavioral health clinics (CCBHCs) and a role out of crisis services that has also been touched on in the Medicaid meetings. Contractual updates for care coordination are in the works, but there may not be the opportunity for re-contracting in the immediate future given delays. Rule changes may be able to help with some policy improvements and we are exploring this further along with other potential directions for more immediate relief for class members.

The preliminary data report on community navigators was delivered to me, showing some anecdotal evidence of potential promise of this model. There were delays in identifying the sites for the community navigators and in hiring staff, but the pilot programs are running, and further data will be reported in the next data cycle. Legislative budgetary requests have been made to support the expansion of this model. There is also a growing recognition that the community navigator program and Medicaid need to collaborate to leverage the strengths of all opportunities for coordination.

There has been a great deal of discussion about the individuals found GEI and the processes needed to discharge them from OSH with PSRB approval. This involves coordination between OSH and OHA and the CMHPs to conduct evaluations, making sure paperwork is submitted to the PSRB and identification of placements. OHA leadership has at times been confused about terminology related to PSRB findings, and recent discussions have concluded that clear terminology and definitions are needed to help track progress in expediting GEI discharges when appropriate.

The Northwest Regional Reentry Center (NWRRC) has continued to provide a state-run contracted service for restoration in the community. The current contractual terms allow for the provision of services to 35 Aid and Assist clients. However, as seen by the data presented above, the contracted service has rarely been fully utilized. According to sources with whom I spoke, the contract has had multiple managers who have not provided rigorous utilization review. In one recent meeting with OHA leadership, OHA staff were unaware of the total number of beds they were contracting for at NWRRC and I had to remind them. In addition, staff at NWRRC presented several cases that appeared to have reached maximum progress in their competency restoration yet had not been referred for evaluation. One individual had been at the facility for approximately two years. These issues are starting to be addressed by Ms. Clarke and her team, but they require ongoing oversight by OHA leadership.

OHA leadership and the governor's office staff have been working together with OSH representatives to meet with legislators and present information regarding funding requests and statutory change requests (See below).

Language pertaining to the County Financial Assistance Agreements has been updated in negotiation with the CMHPs and I have had the chance to review it and provide comments. The OHA leadership is hopeful that the new language will allow CMHPs more flexibility to help meet the needs of the forensic populations.

Overall, the OHA leadership appears to be working diligently, and they have been helpful and willing to engage in dialogue regarding complex issues. At the same time, there have been several recommended items that have been delayed, and meetings where it appears answers are unknown that should or could be known. Frequent meetings have been necessary to keep the work on track to proceed toward delivering the expected outcomes pertaining to my recommendations.

#### **OSH Updates:**

Major updates within OSH include several CMS findings and a recent death, after which Dr. Sara Walker resigned. This occurred in mid-April, and it occurred after Dr. Hathi was on the witness stand at the contempt hearing describing her as a trusted leader on her team. According to news stories and information I received, Dr. Hathi had a meeting several months ago indicating that there were problems at the hospital, yet it is not clear that anything was particularly done to solve those problems. Dr. Walker had been tasked with a nearly impossible workload of being the Chief Medical Officer (CMO) and the Superintendent. There were many conversations I had with her where she noted she was hopeful OHA would hire a permanent superintendent as her goal was to return to the CMO position solely. She often covered units as a treating psychiatrist, making herself accessible to patients and staff. When we toured the facility, it seemed she was well known as she had worked there for many years. She was particularly

responsive in my discussions with her and was a leading voice in advocating for legislative changes that pertain to this case. Given her knowledge of policy and practice at the facility, it is concerning that her resignation was so abrupt. Without someone with her experience, OSH is at risk for having a significant vacuum at the facility. The circumstances for her resignation are somewhat vague and I have not been provided any direct documentation about what happened with the patient who died, who was apparently a civil patient.

With the resignation of Dr. Walker, Mr. Dave Baden was named interim Superintendent with Dr. Bell, previously the director of psychiatry, named as the interim CMO. OHA leadership is working with search firms to identify a team for an interim superintendent and CMO while doing work to enhance the hiring package for more permanent replacements.

At the time of the contempt hearing, revisions to the expedited admission criteria were drafted and I was asked to provide edits to those criteria. The edits were developed to clarify language so that there was less ambiguity about who to refer. I conducted that work with Dr. Walker, and those new expedited admission criteria are now in effect.

Members of the private hospital leadership indicated that there continue to be some quarterly meetings to review the civil admissions issues (at least one this year was canceled). There are major pressures to expand capacity at OSH for civil patients, and ongoing concern by the private hospitals about backlog in their facilities. Mr. Eric Neiman discussed at the contempt hearing his clients' concerns about the emergency rule that prioritizes people in the AA or GEI process for community placements. This will require ongoing tracking to ensure it does not have unintended consequences. Legislative proposals regarding expansion of civil commitment criteria may also impact the dynamics of the behavioral health system and hospital usage.

As noted in the data depiction above, the actual census of the facility has been below the active capacity for far too long. I met with Dr. Walker to review the reasons for this. One of the stated reasons is related to trying to admit individuals very literally based on their order in the queue

on the waitlist. However, there were several counties that would be unable to transport the person to the facility on the day a bed was available, so the bed would be held until the transport occurred. There were also open specialized beds, such as a medical bed at the hospital, where a person with those special needs would have to wait until they were at the top of the waitlist to be placed in that bed. After this was identified, I conferred with Judge Nelson and modified my recommendation for admissions, which the hospital immediately implemented. This recommendation is noted at the end of this report. As of recent data, the hospital has had a census much closer to the 705 listed as the active capacity. Once the hospital demonstrates it is safely able to manage the 705 patients, there can be a further discussion about the remaining “open beds” (37 in total) that are kept open based on operational emergency needs and safety planning as determined during the COVID-19 pandemic.

OSH has been working also to address several areas of corrective action needed for CMS approval as well as Joint Commission.

Dr. Walker had been a leading spokesperson for the legislative work representing OSH, and it is not clear what her resignation did for the credibility of the state leaders working with legislators. Drs. Beckman and Bustos seem to be stepping into conversations with legislators, as well as Scott Hillier, a data analyst at the facility.

The OSH leadership and Mr. Hillier, along with Dr. Alison Bort the Director of PSRB, worked together to create a conditional release data dashboard that is now publicly available. The dashboard is incorporated in the data reported above. One of the findings from this dashboard exercise was the recognition that there were several steps in referral process that were delayed- and a few of those were totally within the control of OSH, others were more under the auspices of OHA CFAA contracts with CMHPs and others related to actions of the PSRB. At each step related to OHA and OSH there were more significant delays. Dr. Walker attempted to address some of the delays that were purely administrative tasks of OSH staff. The outcomes of

interventions to address these delays will continue to be tracked.

### **Forensic Evaluation Services:**

Regarding FES, three additional staff have been hired to conduct evaluations, and there have been further efforts to expand community restoration evaluations. The hiring processes had begun by November 2024, around the time of the 11<sup>th</sup> Report and the status hearing with Judge Nelson. Since that time, FES has been participating more actively in the regular meetings with me and has been closely reporting on progress with the hires and activities related to evaluations. The last person hired was due to start in May 2025.

The number of people awaiting a scheduled evaluation as of 1/22/25, according to Dr. Beckman, was 267. As of 4/23/25, the number of people waiting for a scheduled evaluation was 150. This is a strong improvement, and Dr. Beckman indicated that the number of pending evaluations should continue to go down as the FES staff catch up on outstanding evaluations. Dr. Beckman indicated that after all staff are hired there will be another time study to determine if the workload for the forensic evaluators is correct or needs adjusting.

I had the opportunity to speak with Drs. Guyton and Milkey who conduct forensic evaluations in the community through Northwest Forensic Institute, LLC. They are two of the leading community-based forensic evaluators in Multnomah County, though their work spans many counties across the state. They noted some of the challenges with efficiencies and workload differences between community-based evaluations and the evaluations conducted by FES.

### **Summary of AOCMHP Discussions and Activities:**

The CMHP representatives that I meet with have been very helpful in providing perspectives from the community vantage point. Key themes that emerge continue to be the increased number of people in community restoration and the lack of time limits for that service. Cost

studies have pointed to the need for additional funding with the current growth rate of community restoration.

The CMHP counties where the ECMU has been established have reported some positive gains from that work, with some initial skepticism. Housing barriers, lack of sufficient placements, and lack of local control over placements have been identified as barriers to discharges from OSH.

Anecdotally the CMHP representatives have indicated that the community navigator model seems helpful.

### **Summary of Legislative Efforts Regarding the Mink Restoration Time Limits and Behavioral Health Funding:**

By late fall of 2024 the defendants were working to support legislative changes pursuant to the recommendations and their experience at OSH. The defendants submitted HB 3051 to reflect recommendations previously made in my reports and as ordered in the Federal Courts September 2022 order. This has catalyzed intense discussions with various committees within the legislature, with Representative Kropf taking the lead in organizing the dialogue. There are very strong views about this legislation, and the Oregon District Attorneys Association, the public defenders and OJD are taking strong positions in opposition and have proposed their own remedy and timelines for consideration. Essentially, the thrust of their concerns appears to include their data analysis that shows fewer defendants are getting restored since the implementation of the Mosman order, and their conclusions that this relates to the increase in recidivism among defendants ordered and then discharged from OSH. The state contends that these analyses are at least in part, flawed.

Comparison proposals have been sent to the legislators, including OJD's proposal for a forensic oversight board to help determine timelines for restoration on a more individualized basis by a committee.

Additionally, HB 3715 sponsored by Representative Thomas Andersen, attempts to modify civil commitment statutory criteria, and sets minimum OSH capacity for future civil commitment placements. HB 2470 requested by the Oregon District Attorneys Association, would direct OHA to increase capacity at OSH or elsewhere, and authorizes in-custody jail-based restoration, among other provisions.

Legislation has also been filed in an attempt to secure funding for the services recommended in my 10<sup>th</sup> report. It remains unclear how the legislature will respond to this.

DRO brought out in the contempt hearing that the Mink/Bowman matter did not appear as pressing in the state's legislative testimony at that time. Since the contempt hearing, it appears the state is making strong efforts to explain to legislators the weight of the federal court's decision regarding contempt and what that might mean to the state if fines are imposed.

**Legal Issues in this Matter in this Interim Reporting Period:**

Consistent with the recent history of this case, several court filings have been presented related to this matter since my last report. Most notably, the plaintiffs filed for Contempt after the November status hearing, and a contempt hearing was held in March 2025. In the motion for contempt, DRO sought relief including: 1) monetary contempt sanctions against defendants; 2) modifications of admissions to and discharges from OSH including:

- a. Limit misdemeanor and status offender referrals to the state hospital until compliance is achieved.
- b. Transfer or discharge all aid and assist patients on the Ready to Place list within 30 days.
- c. Discharge PSRB patients on the Ready to Place list within 60 days.

- d. To address the backlog in re-evaluating individuals in community restoration, Defendants shall hire a cadre of evaluators to conduct a re-evaluation of this backlog of patients within 60 days.
- e. Order Defendants to commission an external study by an outside group approved and lead by Dr. Pinals to a) review admissions data, b) develop a statewide process for a more centralized oversight of the forensic mental health system, c) assess projected system capacity to respond to the demand in services and d) make any other findings recommended by Dr. Pinals.

MPD in their response to DROs motion, asked the Court for 1) further admissions limitations as per DRO's motion; 2) Discharge authority of OSH over the courts when someone is deemed to no longer need hospital level of care; 3) GEI discharge improvements; 4) Forensic evaluation capacity increase; 5) Community restoration limits either through legislative fixes or through taking all reasonable steps for creative funding strategies; and 6) an independent audit to determine what capacity has been created and is still needed to foster compliance.

The questions posed by Judge Nelson at the contempt hearing further delineated DRO's stated request for the Neutral Expert role to be modified to a Court Monitor.

In a separate but related matter, Judge Nelson found Marion County and its counsel acted in bad faith by seeking to intervene in DRO's contempt action and imposed a sanction, striking Marion County's filings from the court record.

Other cases that have been filed on related matters include *Berman v. OSH/OHA*, a *pro se* class action lawsuit regarding the individuals found GEI who no longer need institutional care but are waiting for community placement.

On 3/26/25 Judge Nelson ordered that the Court's remedial order be extended until 6/29/25, thereby extending the original Mosman order pertaining to hospital restoration timelines and admissions and its related amendments.

### **Additional Information:**

As a point of reference, there are interested advocates also providing context to the discussions, including Judge James R. Hargreaves who has repeatedly noted that judges are miscalculating the sentence maximum that would relate to legally permissible restoration periods, inappropriately ordering OSH restoration from municipal courts. Joseph Bloom, MD, a renowned forensic psychiatrist, has also published several academic papers on what is happening in Oregon and has focused most recently on the decrease in civil commitments as a major factor in the rise of Aid and Assist cases.<sup>1,2</sup>

### **Conclusions and Recommendations:**

There has been a great deal of activity between my last report and this report. Major conclusions during this period include:

- 1) As of this writing, the state remains out of compliance with the Federal Court's Order to admit individuals from jails within 7 days.
- 2) The state defendants' efforts have resulted in progress toward compliance, with an average waiting time now of 20.7 days and 46 people on the admissions waitlist (down from the 60-70 people that were waiting at the time of the contempt hearing).
- 3) State leaders are working diligently to meet the recommendations delineated on the project tracker, but are delayed on several or have not performed as well on others (such as the community restoration manual, the data report on community restoration, work between ODDS/APD and OHA, full contract monitoring of NWRRC to achieve a

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<sup>1</sup> Bloom JD, E Hansen T, Blekic A. Competency to Stand Trial, Civil Commitment, and Oregon State Hospital. *J Am Acad Psychiatry Law*. 2022;50(1):67-73. doi:10.29158/JAAPL.210055-21; available at: <https://jaapl.org/content/50/1/67.long>

<sup>2</sup> Hansen TE, Bloom JD, Blekic A. The Dramatic Decline of Civil Commitment in Oregon, 1972 to 2020. *J Am Acad Psychiatry Law*. 2022;50(4):533-540. doi:10.29158/JAAPL.220003-21; available at: <https://jaapl.org/content/50/4/533.long>

census of 35 beds). Their work has taken oversight, but with that, they have been responsive.

- 4) New developments at the state hospital (the recent patient death, the resignation of a longstanding clinical leader) call into question its stability. The significant leadership upheaval with plans for interim leadership and questionable responsiveness to the hospital needs by OHA Director Hathi prior to the recent events potentially could have facilitated greater safety for patients there and improved outcomes overall. If it was known that Dr. Walker had too much on her plate with two major positions being held for a prolonged period of time, it is unclear what was being done about this.
- 5) Evaluation backlogs are still too high. Although the work to catch up is progressing steadily, 150 pending evaluations are simply too many. The leadership of FES must do further due diligence to ensure caseloads are appropriate for the staff and determine how to fill the gaps in the more short-term.
- 6) Community restoration is still too long and too disjointed. Work towards finalizing the community restoration manual and data tracking is imperative.
- 7) Hospital restoration timelines currently in place with the Federal Order, and those recommended in my 10<sup>th</sup> report for community restoration (all captured in HB 3051), along with fixes to proposed language regarding competency opinions, should be codified in statute.
- 8) The problems in achieving compliance are multi-faceted, and OHA and OSH alone cannot solve all of them. However, leadership within OHA, OSH, and support from the Governor's Office are critical to continue to prioritize the necessary fixes, which are delineated in the project tracker, and which should incorporate the recommendations set forth in this report, with appropriate timelines.
- 9) Compliance and sustained compliance will require sufficient funding and resources to build out the capacity and hire the staff needed to do the work. Whether that funding comes through the Oregon legislature, or through monetary sanctions by the Federal Court, remains to be seen, but the funding needed must at least address the totality of the recommended strategies that have been laid out across my reports, including those

listed in this report, and the ability to pivot as new recommendations appear to be needed.

Recent reductions in waitlist numbers are, in my opinion, likely attributable to several factors including:

- 1) Opening of a community placement that allowed the discharge of nine people on the GEI discharge ready list;
- 2) Re-configuring how admissions are being processed as described above by my recommendation, accounting for better utilization of approximately 20 additional OSH beds;
- 3) Possible work by the ECMU and the counties and the policy prioritizing the acceptance of forensic patients into community settings;
- 4) Ongoing adherence to the original Mosman order with the timeframes for restoration at OSH shorter than permitted by statute, as well as limitations on some individuals from admission to the hospital per the Mosman order and per expedited criteria factors;
- 5) A relatively steady number of orders over the last three reported months that numbered under 100; and
- 6) Doubled down efforts at achieving compliance with the concerns about contempt.

With all of this in mind, however, a major factor that will determine what will happen with both achieving compliance and sustaining it over time will depend on the willingness of Oregonians, and specifically the legislature, to invest in the behavioral health population. Progress in this case will also depend on the willingness of stakeholders to reconsider when the criminal system is used as the default treatment system under the belief that is always safer and better. The legislative deliberations that are occurring as this report is being written are critical. Yet this is *not a new issue*. Even since my work began on this case in late 2021, the state has had at least four heavily attended workgroups debating the very same thing regarding restoration time frames. Yet none of the proposals seem to examine research, data or other state laws for

examples of what might work for Oregon. And consensus has not been reached. The prosecution and the judiciary (and even the public defenders) seem to be the loudest voices in opposition to timelines that are proposed for restoration services that are consistent with what is currently occurring and would more significantly limit community restoration timelines. In the national dialogue among court leaders, it has been stated that competency restoration should be used to the minimum extent.<sup>3</sup> It seems in Oregon competency restoration is a default option and there is a conflation of how to “help people get treatment” and “attend to public safety”, without focusing on the sole purpose of restoration, which is to help a defendant achieve symptom reduction and increase their functional capacity to competently serve as a pretrial defendant to face criminal charges and be prosecuted.

That there are victims involved in many of the crimes allegedly committed by the people found unable to A&A who are actually guilty of their crimes, and that these victims and members of the public deserve redress and public safety as allowed by the Constitution and state law is without a doubt. However, there are not always person-victims, and many of the crimes for which defendants enter this forensic system are low level crimes in which criminal prosecution is not likely to happen at the end of all these complex processes, making many of them wasteful in terms of resources expended. Relatedly, there is no question that individuals found GEI can at times present an elevated public safety risk and the PSRB is designed to manage some of that.

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<sup>3</sup> “There are days when it feels like I am presiding over dockets of despair – very mentally ill people in the criminal justice system who too often are met by closed doors at every turn. Their families are worn out by the lack of services available. Community programs reject them as too difficult or risky. The public is frustrated and wants these individuals off the sidewalks, out of sight, and out of mind. Sometimes, the only door open to them is the door to the jail. When that happens the competency system often comes into play. Too often as a judge, I too am met with closed doors in the search for the placement and services that will allow them relief from their mental illness and to be restored to competency. At the end of the day, it is cold comfort when the only thing I have to offer is a moment of compassion.” Quote by Task Force Member Judge Nan Waller, Multnomah County, Oregon; Report produced by the National Center for State Courts with quote related to the section entitled, “Use of the Competency Process Should Be Limited and the Restoration Process Should Be Improved”, with a subsection that states, “Actively monitor restoration progress, with appropriate timelines”. This report includes the following recommendation: “Courts and communities should reserve the competency process, including evaluation of competence to stand trial, for defendants who are charged with serious crimes. Others, especially individuals charged with misdemeanors and assessed as low risk to recidivate and whose clinical conditions are not likely to substantially improve (e.g., individuals with dementia) should be diverted to treatment.”, see pages 25-26, available at: [www.ncsc.org/\\_\\_data/assets/pdf\\_file/0031/84469/MHTF\\_State\\_Courts\\_Leading\\_Change.pdf](http://www.ncsc.org/__data/assets/pdf_file/0031/84469/MHTF_State_Courts_Leading_Change.pdf)

However, there are also rights related to liberty and the Americans with Disabilities Act that need to be balanced as placement decisions are made for the individuals caught in these systems and these points have been raised in litigation against over institutionalization of individuals in Oregon with disabilities, whether they are in criminal/forensic processes or civil processes. Thus, remedies are complex and must be balanced.

National data supports that individuals with mental health conditions are disproportionately directed toward criminal systems, that risk is over-predicted, and this is compounded by health-related social needs that come with poverty, houselessness as well as trauma histories and a host of other factors. In addition, the scientific literature is clear that factors other than straight mental illness, including substance use, is more associated with recidivism and challenges for this population. The legislative testimony I have heard repeatedly sites the shortened restoration times as the cause of decreased percentages of people being restored and that case dismissals are the cause of increased recidivism. First, many of the individuals discharged unrestored are simply moved to a different location for restoration, and their ultimate outcomes are not fully tracked. Also, the conclusion that is drawn is that when cases are dismissed, “recidivism rates rise” is an assertion without foundation. Although there may be correlation, the causation element has not been proven.

Recidivism can be accounted for by many factors, including a police officer’s discretion, prosecutorial charging authority, and the quality of defense attorneys in identifying ways to defend their clients. An alternative theory is also that the people who have been in the Aid and Assist processes are known and are more likely to be re-charged rather than diverted away from criminal case processing.

There was a strong example of this recently for one case where a woman who committed a tragic offense while impaired by severe psychosis was found never able to aid and assist and was committed under the dangerous person 701 criteria. She was treated and then became ready and fully authorized to be placed in a community setting. She was discharged from the PSRB

oversight, only to be re-indicted on the underlying offense, and ordered for repeat competency evaluations. She was found able by two evaluators, and is currently in jail, after years in the hospital, awaiting a GEI finding, only to start the process toward discharge again when there have been no untoward incidents and only legal processes that have required prolonged institutional stay. This type of legal process is convoluted. Although I have not clinically reviewed her case, this is the type of situation where general, wholistic support of an individual's psychiatric needs in a proper less restrictive setting may be most helpful for their stability and risk mitigation. This type of issue has been seen as a national trend. In fact, data suggests that arrests and re-prosecution are their own contributors to the risk of disengagement from services.<sup>4</sup> Thus, there are many other factors that could explain what happens with case outcomes and having a simplistic cause and effect formula does not take into account the necessary nuances.

From the time my work began on this case both the state and the plaintiffs have worked with me to achieve two broad goals- to facilitate that the state achieve compliance with the seven-day admission rule, AND to help prioritize expansion of appropriate services across a continuum of care that will help the state sustain compliance and decrease the penetration of the population of people with mental disorders in jails when that is not the appropriate place for them to be.

As delineated in my 10<sup>th</sup> report, I have met with the defendants across many meetings to review all prior recommendations and to clarify where the state's activities have been and where the state is regarding timely completion of milestones delineated in the project tracker. This has required much discussion as there were several delays that had to be explained. Many of the delays were not entirely in the control of OHA (e.g., a data sharing agreement with OJD has taken a particularly long time), but leadership, in particular Ms. Ebony Clarke, Dr. Beckman, Dr. Bustos, and previously Dr. Walker, and members of the Department of Justice who support

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<sup>4</sup> Hwang YIJ, Albalawi O, Adily A, et al. Disengagement from mental health treatment and re-offending in those with psychosis: a multi-state model of linked data. *Soc Psychiatry Psychiatr Epidemiol.* 2020;55(12):1639-1648. doi:10.1007/s00127-020-01873-1; <https://pubmed.ncbi.nlm.nih.gov/32390094/>

OSH and OHA, have been responsive to the direction that is needed. Progress reports continue to be completed monthly, and ongoing review of the project tracker will help keep the work moving forward.

To the question of whether the state should be held in contempt, this is ultimately a legal finding. There are data showing the state remains out of compliance and that this has occurred for months. When I wrote my report in November, I had anticipated that if the situation was not remedied within three to six months, that a contempt finding might yield further pressures to facilitate action by the state. A finding of contempt does not achieve a “quick fix” to a slow system (as noted by MPD Plaintiff Mr. Merrithew in his commentary to the court). Thus, given the current progress, and the potential for the legislature to take proper and swift action itself, the Court may cautiously wish to see if the numbers continue to improve.

Regarding the several the remedies sought by plaintiffs that were presented at the contempt hearing, I stand by my opinions offered at that hearing about the pros and cons of those remedies. I further delineate my current recommendations below for the Court’s consideration.

- 1. Continuation of the original Mosman Order with limitations on admissions to OSH and time limits for restoration services.** This is currently set to expire, 6/29/25 extended by Judge Nelson on 3/26/25. Until there is a legislative remedy, any withdrawal of this order will immediately result in a regression away from compliance. I recommend that it continues to be in effect for another year, during which time the fluctuations in the system will be able to be monitored.
- 2. Modification of OSH Admissions and Discharge Processes.** As articulated to Dr. Sara Walker on 4/8/25 it is imperative that the hospital use hospital bed space to the full extent of its managed capacity and potentially expand use of other available beds in the facility when it appears safe to do so. As such, I now recommend the following:

- a. OSH should continue to admit people based on the order of their placement on the waitlist, except in those circumstances where for logistical reasons they cannot be admitted (e.g., sheriffs unwilling to transport the day a bed is available, female bed vs male bed, medical beds that are best filled with individuals with more medical needs, etc.), in which case some variations to that sequencing would be reasonable to get your census up to managed capacity, with those variations still considering the date of the original order for placement (e.g., two competing similar female bed needs would warrant having the first one on the list to come in first). In addition, the other exception to the admission order would be if there are expedited admissions that need more immediate admission for clinical reasons per the protocols that have been established- these would be able to be admitted before all others if the criteria are met.

The hospital has already implemented this recommendation, but I delineate it here for the Court's attention.

- b. Admissions could be further restricted such that misdemeanor defendants not be eligible for OSH admission for restoration purposes (consistent with remedies suggested by both plaintiffs). The concern with this recommendation at this time is that there could be an increased rate of more serious charges being levied, and as such, with the numbers already turning in the right direction, this may not be a necessary step.
- c. Plaintiffs from DRO asked for discharges to be mandated within timeframes. I do not agree with that remedy as that may create unsafe clinical situations for patients. Instead, I continue to recommend MPD's remedy that discharges be fully under the authority of OSH in collaboration with CMHPs pursuant to recent rule changes.

- d. Admissions that are determined to not need hospital level of care within 10 days should be examined and expedited discharge processes should be developed.

**3. Further review of recent patient needs, reporting on OSH activities and leadership recruitment.** OSH should be required to regularly report to the Neutral Expert any sentinel events at the hospital, and provide status updates regarding the hiring of a permanent CMO and Superintendent. OHA should immediately restructure these positions to ensure salaries are competitive to hire the most qualified staff, and report those efforts to the Neutral Expert. The Neutral Expert should in turn be authorized to bring in additional consultants or make recommendations to improve aspects of services at OSH that impact compliance and case flow through the hospital.

Additionally, the Governor should further examine more closely why OSH was not given additional resources to help support patient care and the existing leadership at OSH surrounding recent struggles at the hospital. There may be a deeper remedy needed beyond the resignation of Dr. Walker. Further details regarding care delivery at OSH should be transparently shared with the Neutral Expert who may make further recommendations based on her training and experience.

**4. Development of Medicaid Remedies to Facilitate Continuity of Care for Mink/Bowman Class Members.** It is concerning that Medicaid re-contracting is being delayed, and it is clear that Medicaid policy planning currently does not have a “quick fix” for the Aid and Assist population or the GEI population. To maximize understanding of the relationship between Medicaid services and the forensic population needs and develop more expedient pathways to maximize continuity of care additional Medicaid recommendations include:

- a. OHA should establish a system for identifying who is funding services for individuals in the Aid and Assist processes, especially considering the Optumas

data showing that a significant portion are not receiving Medicaid funding. In addition OHA Medicaid should begin tracking individuals in the Aid and Assist program otherwise eligible for Medicaid as they leave jail or prison or the Oregon State Hospital (OSH) or residential treatment facilities (RTFs) with the capacity to report how many of these individuals are transitioning out to their communities from these settings, whether they are accessing Medicaid benefits, how long it takes to get them enrolled, the degree to which they are utilizing emergency departments, and how often and how quickly are they reentering the criminal justice system.

- b. In addition, the proposed tracking system/Medicaid case management model for Medicaid-eligible individuals in the Aid and Assist program could facilitate rapid re-enrollment into CCO coverage for these individuals as they are leaving jails/prisons or the OSH or RTFs. Recent [CMS guidance](#) on “Accessing Enhanced Federal Medicaid Matching Rates for State Information Technology Expenditures to Improve Access to Mental Health and Substance Use Disorder Treatment and Care Coordination” could potentially help the state identify some federal funding to support development of this tracking system.<sup>5</sup> The state should reach out to the CMS state Medicaid lead for Oregon to explore how this authority for enhanced federal administrative match could be accessed to support improved care coordination and eligibility and enrollment activities for Aid and Assist individuals otherwise eligible for Medicaid as they are leaving jails and prisons, OSH, and RTFs.
- c. OHA should explore all means to implement a Medicaid presumptive eligibility policy and process to designate individuals in the Aid and Assist program as presumptively eligible for Medicaid program immediately after they leave

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<sup>5</sup> <https://www.medicaid.gov/federal-policy-guidance/downloads/cib06142024.pdf>

jails/prisons, OSH, and RTFs and designate these facilities as qualified entities for purposes of turning on Medicaid coverage as part of the discharge process from these settings. CMS [Implementation Guide](#) on Medicaid presumptive eligibility refers to jails as well as other facility types as potential “qualified entities” for determining presumptive eligibility.<sup>6</sup>

- d. OHA should coordinate to implement overarching discharge policies and requirements to assist the Aid and Assist population as they are leaving OSH and carceral settings and include requirements that these settings access the MMIS system to restart enrollment in the CCO that the person was previously enrolled in before being incarcerated or committed to OSH upon the individual’s exit from one of these facilities. If access to MMIS is not allowable or practical by these types of settings, OHA should designate a Medicaid agency division charged with assisting the discharge process and reenrollment of individuals in the Aid and Assist program into the same CCO that they were previously enrolled in immediately upon leaving jail/prison, OSH, or RTFs.
- e. The discharge requirements for jails/prisons, OSH, and RTFs should also require that these settings connect individuals involved in the Aid and Assist program with the Community Navigator program as they are leaving these settings.
- f. To further support individuals involved in the Aid and Assist program as they reenter their communities, some of the new 1915(i) case managers should be designated as specializing in helping the Aid and Assist populations connect with home and community-based services including rehabilitative and social support services. These case managers should also be required to collaborate closely with the Community Navigator program.

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<sup>6</sup> [www.medicaid.gov/resources-for-states/downloads/macpro-ig-presumptive-eligibility-by-hospitals.pdf](http://www.medicaid.gov/resources-for-states/downloads/macpro-ig-presumptive-eligibility-by-hospitals.pdf)

- g. Delays in CCO re-contracting require further examination, and remedies that are more immediate from Medicaid to enhance continuity of care should be further explored and pursued as available, even if it means bringing state general fund dollars to help with bridge funding within the CCO and CMHP efforts.
- 5. **Ongoing Advocacy for needed legislative change and funding.** OHA and OSH leadership, along with the Governor's office, should continue to advocate for legislative appropriations for community services and for time limits for restoration in accordance with my 10<sup>th</sup> report recommendations. This is critical work and is necessary for if the state is to maximize its ability to solve its own challenges with less federal court oversight. As noted above, timeframes for restoration are critical, but also are only one piece of the remedy. Compliance cannot be achieved without sufficient and necessary funding and resources.
- 6. **GEI Process Improvements.** As noted in my 10<sup>th</sup> report, and consistent with MPD's motion for relief, there are too many delays in GEI discharges. Although OSH did modify and shorten their expected time frames for some of the steps for approval that were within their administrative control, none of the steps meet the expected timeframe, and the closest one that does is related to PSRB processes. OHA and OSH processes are still delayed. There is also confusion at times among state leaders about how the processes work. Thus, I recommend the following:
  - a. OHA and OSH should work together, along with the PSRB leadership, to create a clear language for the steps needed to discharge individuals found GEI from OSH. All documentation should use similar labels, including labels seen on the new conditional release data tracker.

- b. OHA and OSH should demonstrate at least a 20% improvement in the time it takes for discharge of GEI patients once identified by the treatment team as discharge ready, and this improvement should be achieved within three months.

**7. Establishment of a community-based formalized forensic evaluation and restoration service and increased access to evaluators to reduce backlog in the short term and eliminate backlog in the long term.** Consistent with remedies suggested by both plaintiffs, more evaluators are needed to complete evaluations in the short term and improvements are needed related to community-based evaluation services.

- a. Forensic Evaluation Services should expedite the completion of the 150 pending evaluations within three months and staff should be contracted if additional evaluators are needed beyond the current staffing complement.
- b. OHA should establish an office for community forensic services whether that becomes part of OSH FES or is managed through OHA. This will be critical to help maximize expertise in building out evaluation systems and operations for community restoration that includes regular competency updated evaluations as supported in statutory proposals. These should begin regardless of the existence statute and the state should be compelled to ensure that they are completed, so that individuals do not languish in community restoration services for unlimited periods of time.
- c. Given the prolonged delays in receipt of OJD's GAINS center report recommending options for forensic evaluation services, I withdraw my earlier recommendation to rely upon it.

**8. Community restoration improvements.**

- a. Pursuant to my prior recommendations, the community restoration manual should be completed and timelines for its completion delineated in discussion with the neutral expert. It will require updates to ensure it meets the specificity outlined in my Second Report.
- b. OHA should revisit options and develop a mechanism for paying only for the portion of restoration services that occur within the community restoration timelines outlined in HB 3051. This is consistent with the MPD proposed remedy to the Court. Any other time needed should not fall to OHA or its contracted entities. Individuals in the Aid and Assist process, however, should continue to receive medically necessary services as per their usual entitlements when they are Medicaid eligible or medical services according to Medicare or private payers. This will require a delinking of what community restoration services are that are not Medicaid-reimbursable and those that are, and a concomitant revisiting of rules in the CFAA and Medicaid discussions,
- c. Training efforts should continue to be pursued by OHA and OSH to help educate willing community partners on the activities and opportunities for services for individuals eligible to receive public services.

**9. Bed Capacity tracking and commissioning a study to examine community capacity and needed improvements overtime specific to the forensic population.**

- a. I support the recommendations of both MPD and DRO for an audit and study of bed capacity and recommend it occur under the Neutral Expert's guidance.
- b. OHA should examine its bed tracking website and ensure its accuracy and consistency. The staff responsible for uploading data and OHA leadership should

confer and, in consultation with the Neutral Expert and perhaps others, review why there are discrepancies in bed capacity data across time and how to rectify data reporting.

- c. OHA should report on regular progress regarding capacity expansion planning as recommended in my 10<sup>th</sup> report.

**10. Improvements in contract management of NWRRC.** It has become clear during this interim period that contract management of NWRRC has not yielded utilization of the site to the full agreed upon capacity of 35 beds and OHA leadership has not always been clear on what capacity they were supposed to achieve. Several recommendations in this area are in order.

- a. Contract management of NWRRC should be rigorously monitored at high levels of OHA with regular data reports to the Neutral Expert.
- b. Utilization management needs to include re-evaluations so that individuals whose AA status changes are not left to languish at NWRRC.
- c. Close oversight of expansion plans with construction design that is clinically oriented is imperative. The Neutral Expert should be provided with regular architectural updates and planning information. This should include an examination of the potential need (and funding for) a clinical director who has credentials that can work with people with complex behavioral health needs, conducting assessments when needed and developing policy and practices that are infused with clinical approaches.

**11. Focus on discharges through expansion of ECMU activities for AA and GEI populations.**

Although the ECMU is just beginning to develop its mechanisms of action, this type of

oversight appears to be positive, and its efforts should expand to additional counties. The RTP list and the Conditional Release Ready list must be reduced. *Over 130 people are identified at OSH as not needing OSH services.* Oregonians and the legislature should take note of that simple data point. Notwithstanding the concerns about over-reliance on institutions in violation of the ADA, at approximately \$650,000 per year, the OSH service should be limited to those who need it.

**12. Coordination between state agencies to enhance access to specialized services for those**

**who need it.** Exploration of all types of services and throughput throughout the system, including work with SUD and ODDS and APD services should be built into the work. It is not acceptable that there remain silos, and that one agency does not engage with another related to the AA and GEI processes. This has been an issue for far too long. I recommend Behavioral Health leadership in the Governor's office lean into this issue. As per previously delineated recommendations, work between SUD service managers, ODDS, APD and OHA should be collaborative and built out further, with a breakdown of existing silos to prioritize appropriate services for this population.

**13. Community Navigator continuation and expansion.**

The community navigator program must be expanded. Continuity of care and support during transitions are critical if individuals in AA and GEI processes are to maximize their stability and move toward recovery and decreased engagement in problematic activities, including criminal activities and substance use.

**14. Ongoing review of implementation status of prior and current recommendations.**

Work should continue in earnest to review and complete recommendations as delineated in the project tracker. A metric should be added to determine whether the activity is complete, partially complete, or incomplete by the deadline to help track activity and provide a better metric for the court in determining compliance and contempt issues.

The recommendations above should be cross-walked and added to the project tracker (unless they are already identified) and incorporated in the work of OHA and OSH in consultation with the Neutral Expert. Timelines for accomplishing these steps should be identified in the tracker with the Neutral Expert input and that of plaintiffs.

I appreciate the hard work of the state leaders with whom I have regular contact, and the staff working daily for OSH, OHA, providers, CMHPs and other organizations to facilitate recovery and stability for individuals in the AA and GEI processes and promote public safety. The energy surrounding this *Mink/Bowman* matter, including the high level of importance placed upon it by the Governor, is what is undoubtedly helping turn things in a better direction. With that in mind, as I have said before, it is critical that the work continues with the same degree of effort to achieve compliance and even after compliance is achieved again.

I reiterate that the *Mink/Bowman* class members are the focus of this case, and their needs must be met in the least restrictive environment possible and in accordance with the Constitution and the related Federal Court rulings.

**Respectfully Submitted,**

A handwritten signature in cursive script, reading "Debra A Pinal".

Debra A. Pinal, M.D.

Neutral Expert, *Mink/Bowman*